TRIALSCREEN

Sponsor Content Pipeline

Fully-Written B2B Content for Pharma, CRO, Site and Institutional Audiences

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CONFIDENTIAL

TrialScreen Sponsor / B2B Content Pipeline

Voice principles in force throughout this document: - Declarative openings -- lead with the fact, not the company - Insight-first structure: data point -> implication -> TrialScreen connection - Humble authority: "We're seeing..." not "TrialScreen is the leader..." - Credit collaborators generously - No efficacy claims for any investigational drug; use "investigational" qualifier - Series-based content; singletons don't compound

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> - Series-based content; singletons don't compound

Part 1: Publishing Calendar -- First 90 Days

Start date: Monday, Week 1 of Q2 2026 (adjust to actual launch Monday)

Week	Monday	Wednesday	Friday	Channel Focus
1	SP-LI-001 "Patients Are Searching" (Hugo, LinkedIn)	SP-LI-002 "53.9%" data insight (Hugo, LinkedIn)	Email Seq A -- Email 1 sent to pharma cold list	LinkedIn thought leadership launch
2	Blog publish: "The Patient Is Already Searching" flagship article (trialscreen.inc/insights)	SP-LI-003 Trial Discoverability Index intro (Emma, LinkedIn)	Email Seq B -- Email 1 to CRO warm list	Article syndication + LinkedIn long-form
3	SP-LI-004 State of Scleroderma teaser (Hugo, LinkedIn)	Long-form publish: Trial Discoverability Index Q2 2026 report	Email Seq C -- Email 1 to institutional/site warm list	Flagship report launch week
4	SP-LI-005 Sponsor case study #1 (Elisa, LinkedIn)	SP-LI-006 Elisa Cascade IWD post (Elisa, LinkedIn)	Email Seq A -- Email 2 to pharma cold list	Case study + IWD amplification
5	Long-form publish: State of Scleroderma 2026 (trialscreen.inc/insights)	SP-LI-007 Registry hygiene as equity (Hugo, LinkedIn)	Email Seq B -- Email 2 to CRO warm list	Condition report launch week
6	SP-LI-008 FLOW 72-hour rule data (Melissa, LinkedIn)	Webinar 1 promotional post -- "Mastering Recruitment 2026" (reg open)	Email Seq C -- Email 2 to institutional list	FLOW product education + webinar promo
7	SP-LI-009 Trials@YourOrg health systems post (Emma, LinkedIn)	Email Seq A -- Email 3 (case study teaser) to pharma cold list	Long-form publish: Sponsor Case Study #1 (trialscreen.inc/insights)	Case study distribution week
8	SP-LI-010 APAC opportunity (Nick Lam, LinkedIn)	SP-LI-011 ResearchFriends diversity data (Hugo, LinkedIn)	Email Seq B -- Email 3 to CRO warm list	Geographic expansion + diversity themes
9	**Webinar 1: "Mastering Patient Recruitment 2026"** (live, co-hosted Syneos Health)	Post-webinar recap post (LinkedIn)	Email Seq C -- Email 3 to institutional list	Webinar week -- all channels activated
10	SP-LI-012 ACRP 2026 Orlando recap (Elisa, LinkedIn)	Webinar 2 promotional launch -- "Research Friends: Community as Retention"	Email Seq A -- Email 4 (FLOW explainer) to pharma list	Conference debrief + next webinar promo

Week	Monday	Wednesday	Friday	Channel Focus
11	SP-LI-013 Older adults in digital recruitment (Hugo, LinkedIn)	Email Seq B -- Email 4 (tech integration details) to CRO list	Sales enablement: distribute REFER tier one-pager internally + to partners	Data insight week
12	SP-LI-014 FDA OPDP enforcement primer (Hugo, LinkedIn)	SP-LI-015 Year-in-numbers mid-year accountability (Hugo, LinkedIn)	Email Seq A -- Email 5 (offer: enrollment diagnostic); Email Seq B -- Email 5 (offer: partnership call); Email Seq C -- Email 4+5 sent	90-day close: compliance leadership + conversion push

Channel key:

- **LinkedIn:** All 15 posts published via named leader profiles (not company page -- personal > brand)
- **Email:** Three simultaneous nurture sequences running in parallel to segmented lists
- **Insights hub (trialscreen.inc/insights):** 4 long-form pieces + 3 case studies published sequentially
- **Webinar series:** Webinar 1 in Week 9; Webinars 2-6 scheduled Weeks 13-26 (Q3)

Part 2: LinkedIn Posts -- 15 Fully Written Pieces

SP-LI-001

Content ID: SP-LI-001

Author: Dr Hugo Stephenson, CEO, TrialScreen

Series: Patients Are Searching

Target Segment: Pharma (VP Clinical Operations / Head of Patient Recruitment)

Posting Guidance: Post Monday morning Week 1 -- anchor post for entire campaign. Pin to profile for 30 days.

Full Post Copy:

Patients are searching. Right now.

Not waiting for a doctor referral. Not waiting for a pharmaceutical ad to find them. Searching -- on their phones, in hospital waiting rooms, at 2am when the fear gets loudest.

We track search behaviour across trialscreen.org, which lists more than 120,000 actively recruiting studies in over 200 countries. The signal we're seeing is consistent and it's been consistent for years: patients with serious and rare conditions are far more motivated to find trials than the clinical research industry typically assumes.

And yet enrollment remains the single biggest bottleneck in drug development. Roughly 85% of trials fail to meet their enrollment targets on time. Roughly 80% of trials experience delays. The average Phase III trial takes 15 years from investigational compound to approval.

There's a gap here that doesn't add up.

If patients are searching -- and the data says they are -- why are so many trials struggling to find them?

Part of the answer is discoverability. Most trial listings are written in protocol language that no patient without a medical degree can parse. Search algorithms surface results that match clinical terminology, not the plain-language terms patients actually use. A patient searching "joint pain that won't go away" is unlikely to find a trial listed as "a randomized, double-blind, placebo-controlled study of a novel JAK inhibitor in adults with active rheumatoid arthritis."

Part of the answer is trust. Patients have been burned by healthcare systems. Communities with historically low participation rates -- Black, Indigenous, Latino, older adult, rural -- have documented reasons to be cautious. Recruitment that feels transactional will fail with these populations every time.

And part of the answer is speed. When a patient does find a trial and reaches out, the average response time from sites is measured in days or weeks. Our own data from FLOW, our pre-screening product, consistently shows that patients who are not contacted within 72 hours of expressing

interest are effectively lost. They found another option, or they gave up.

The good news: all three of these problems are solvable. We're not at the mercy of fate here.

What I've been thinking about lately is whether the industry is asking the right question. The question "how do we find more patients?" may be less useful than "how do we make it easier for patients who are already looking to find us?"

That reframe changes everything -- the technology you invest in, the language you use on listings, the speed at which you follow up, the trust infrastructure you build before someone is ever eligible.

I'll be writing about each of these over the coming weeks. This is the first post in a series I'm calling Patients Are Searching.

If this resonates -- or if you're seeing something different in your trials -- I'd genuinely welcome the conversation.

Suggested Visual: Clean data graphic showing the gap between patient search volume and trial enrollment rates. Dark navy background, TrialScreen brand palette. Text overlay: "85% of trials miss enrollment targets. Patients are searching anyway."

Hashtags: #ClinicalTrials #PatientRecruitment #RareDiseaseResearch #TrialDiscoverability #DrugDevelopment #ClinicalResearch

CTA: "Follow for the Patients Are Searching series. And if you're a sponsor or CRO looking at enrollment gaps -- we'd like to show you what we're seeing. Start at trialscreen.inc"

SP-LI-002

Content ID: SP-LI-002

Author: Dr Hugo Stephenson, CEO, TrialScreen

Series: Trial Discoverability Index

Target Segment: Pharma (VP Clinical Operations / Head of Patient Recruitment)

Posting Guidance: Post Wednesday Week 1, two days after SP-LI-001. References prior post to build series continuity.

Full Post Copy:

53.9%.

That's the proportion of patients with rare diseases who first learned about their clinical trial options through an online search -- not through their physician, not through a patient advocacy group, not through a hospital referral system.

We're going to publish the full data in our inaugural Trial Discoverability Index next week. But I wanted to lead with this number because it challenges a core assumption embedded in how clinical trial recruitment is typically funded and designed.

The dominant model of trial recruitment was built for a world where the physician is the gatekeeper. Sponsor outreach goes to investigators. Investigators refer patients. The patient is a passive recipient

of information. In that model, spending on patient-facing digital infrastructure feels optional -- a nice-to-have, not a strategic necessity.

53.9% says that model is broken. Or at least: it's incomplete.

For rare diseases in particular, the patient is often the expert in the room. Someone living with scleroderma, or a paediatric cancer, or an ultra-rare metabolic condition has typically done more research about their disease than any generalist physician they've seen. They found their support community online. They found their specialist online. And yes -- they're finding their clinical trials online.

What follows from this?

First: your trial listing is a patient touchpoint. The way it's written, the search terms it's indexed against, the trust signals it carries -- these matter. Protocol-language listings written for IRB submission are not the same as listings written to be found by a patient at 11pm.

Second: the speed of follow-up after a patient expresses interest matters enormously. Digital search creates intent. If that intent isn't met with a warm, timely response, it evaporates. We're seeing 60%+ drop-off in patient interest when first contact doesn't happen within 72 hours.

Third: the patient search journey is measurable. We can tell you -- for a given condition, in a given geography -- what patients are searching for, what language they use, and how many potential participants are actively looking right now.

Full Trial Discoverability Index launches Wednesday next week. The methodology is transparent; the data is real.

If you'd like to be notified when it publishes: trialscreen.inc

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Suggested Visual: Single large number "53.9%" in TrialScreen brand typography on a clean background. Subtext: "Patients with rare diseases who first discovered trial options via online search. Source: TrialScreen Trial Discoverability Index Q2 2026."

Hashtags: #TrialDiscoverabilityIndex #RareDiseaseResearch #PatientRecruitment #ClinicalTrials #DigitalHealth #EvidenceBasedRecruitment

CTA: "Follow to be notified when the full Trial Discoverability Index publishes next week. trialscreen.inc/tdi"

SP-LI-003

Content ID: SP-LI-003

Author: Emma Burrows, Head of Scientific Engagement, TrialScreen

Series: Trial Discoverability Index

Target Segment: Pharma / Institution (Research Directors)

Posting Guidance: Post Wednesday Week 2, same day the Trial Discoverability Index report goes live. Link directly to the report.

Full Post Copy:

Today we're publishing the inaugural Trial Discoverability Index.

I want to explain what it is, why we built it, and why it's different from the market research reports you've probably already seen.

Most reports on clinical trial recruitment describe the problem -- enrollment delays, screen failure rates, underrepresentation of key populations. They're useful. But they're largely retrospective. They tell you what failed in trials that already ran.

The Trial Discoverability Index is prospective. It measures what's happening right now in the search behaviour of patients and caregivers who are actively looking for trials. We track query volume, query language, and conversion signals -- meaning, what searches lead to genuine referral interest -- across more than 120,000 trials listed on trialscreen.org.

What we're reporting in Q2 2026:

The five therapeutic areas with the highest patient search demand relative to active trial supply -- meaning conditions where patient intent significantly outpaces available trial opportunity. These are the areas where, if a new trial opens, digital demand is already there.

The five trial listing formats with the highest conversion from search to referral inquiry. Spoiler: plain language matters more than we even expected.

A new metric we're calling the Discoverability Score -- a composite of listing completeness, plain-language accessibility, search indexation quality, and response time data where we have it.

Geographic analysis: where demand is concentrated, and where it's being systematically underserved.

The methodology is fully described in the appendix. We use real search and engagement data from trialscreen.org, supplemented by search trend analysis. We are not claiming this is a population-level epidemiological study -- it is a demand signal report from a large, patient-initiated dataset.

Our goal in publishing this quarterly is to give sponsors, CROs, and sites a current, actionable intelligence layer that doesn't exist anywhere else. Not to sell something with it -- to move the field.

The report is free to download.

Read it, challenge the methodology, use the data. That's the point.

-> trialscreen.inc/tdi

Suggested Visual: Cover image of Trial Discoverability Index Q2 2026 report -- clean, data-forward design, with the TrialScreen logo and a headline graphic showing the top five high-demand therapeutic areas.

Hashtags: #TrialDiscoverabilityIndex #ClinicalResearch #PatientRecruitment #RareDisease #DigitalRecruitment #OpenData

CTA: "Download the full Trial Discoverability Index Q2 2026 free at trialscreen.inc/tdi -- and follow for Q3 when it publishes in July."

SP-LI-004

Content ID: SP-LI-004

Author: Dr Hugo Stephenson, CEO, TrialScreen

Series: State of Recruitment (Condition Reports)

Target Segment: Pharma / Patient Advocacy Group Directors

Posting Guidance: Post Monday Week 3, teasing the Scleroderma report before it publishes Wednesday.

Full Post Copy:

Scleroderma affects approximately 2.5 million people worldwide. In Australia, where TrialScreen has done significant work with Scleroderma Australia and their member community, the figure is roughly 20,000 -- and those patients are, almost universally, extraordinarily motivated to find research options.

The challenge isn't finding patients. The challenge is a combination of geographic fragmentation, small trial populations, and the complexity of scleroderma's clinical picture -- which spans pulmonary, renal, gastrointestinal, and musculoskeletal systems, making multi-site eligibility matching genuinely difficult.

We've been sitting with these dynamics for a while, alongside our colleagues at Scleroderma Australia, and we wanted to do something useful with what we know.

On Wednesday we're publishing the State of Scleroderma Clinical Research 2026 -- the first in a series of condition-level reports we'll produce quarterly. It covers:

-> The current global trial landscape: how many trials are actively recruiting, in which countries, for which manifestations

-> Patient search behaviour specific to scleroderma: what they're searching for, what language they use, how they describe symptoms

-> Enrollment patterns: where trials struggle most, and where demand exists but isn't being captured

-> The advocacy infrastructure: how patient advocacy groups are (and aren't) integrated into recruitment pathways

-> Recommendations for sponsors, sites, and advocacy partners

This is not a report about TrialScreen. It's a report about the field -- written with Scleroderma Australia and grounded in patient experience.

We'll be publishing condition reports for several rare disease areas over 2026. If your therapeutic focus overlaps with areas of high unmet need and thin trial infrastructure -- and there are many -- I'd invite you to speak with us about how we might structure the data for your indication.

Publishing Wednesday. Link in first comment.

Suggested Visual: Split image -- Scleroderma Australia logo alongside TrialScreen branding, with text: "State of Scleroderma Clinical Research 2026. Publishing Wednesday."

Hashtags: #Scleroderma #RareDisease #ClinicalTrials #PatientRecruitment #SystemicSclerosis #ClinicalResearch

CTA: "Follow for Wednesday's publish. If you're a sponsor or site working in rare connective tissue disease, DM us or visit trialscreen.inc"

SP-LI-005

Content ID: SP-LI-005

Author: Elisa Cascade, Chief Growth Officer Americas, TrialScreen

Series: Sponsor Spotlight / Case Studies

Target Segment: Pharma (VP Clinical Operations)

Posting Guidance: Post Monday Week 4. Pair with full case study publication on the website same week.

Full Post Copy:

I want to share a result that I think is worth talking about openly.

One of our pharma sponsor partners -- a mid-size oncology-focused company running a Phase II study in a rare haematological indication -- was sitting at 41% of target enrollment after 14 months. That's not unusual. But 14 months in, with the window narrowing, it's a difficult position.

They came to us not because they wanted a vendor to add to the mix. They came because their CRO had already tried two conventional recruitment approaches and both had underperformed. What they needed was a different input, not more of the same.

Here's what we did, and I want to be specific because I think specificity is what makes a case study useful:

We started by auditing their trial listing. The existing listing was written for a clinician audience -- detailed, accurate, and essentially invisible to a patient searching in plain language. We rewrote it collaboratively. Same trial, same eligibility criteria -- but written so a patient who'd been living with this condition for years could understand it in under three minutes.

We paired that with a targeted digital outreach through trialscreen.org and a ResearchFriends community activation in two of the highest-prevalence regions. We also implemented FLOW pre-screening at three sites -- meaning that when a patient expressed interest, our team handled the initial eligibility check digitally and delivered warm, pre-screened referrals to the site within 72 hours.

Enrollment increased 38% over the following 16 weeks compared to the 16 weeks prior.

The full case study -- with site breakdown, pre-screening conversion data, and the listing optimisation methodology -- is published today at trialscreen.inc/insights

I'm posting this because 38% is meaningful. It's not magic; it's structural. The trial didn't change. The patient population didn't change. What changed was how easy it was for patients to find the trial, and how fast they were followed up when they did.

Suggested Visual: Clean infographic showing enrollment trajectory -- before/after curve with Week 0 marked as "TrialScreen activation." Key callout: "+38% enrollment in 16 weeks."

Hashtags: #ClinicalTrials #PatientRecruitment #OncologyResearch #RareDisease #TrialEnrollment #ClinicalOperations

CTA: "Full case study with methodology at trialscreen.inc/insights -- or DM me if you want to talk through whether a similar audit makes sense for your program."

SP-LI-006

Content ID: SP-LI-006

Author: Elisa Cascade, Chief Growth Officer Americas, TrialScreen

Series: Patient Voices / Standalone

Target Segment: All segments (IWD broad reach -- pharma, CRO, site, institution)

Posting Guidance: Post Wednesday Week 4. Time to International Women's Day or Women in Clinical Research awareness moments. Tag relevant professional communities.

Full Post Copy:

What 30 years in clinical research has taught me.

I came into this field thinking the science was the hard part. I was wrong. The science is the reason. The people are the work.

Thirty years across pharma, CRO, and now TrialScreen -- and the consistent truth I keep returning to is this: the reason trials fail to enroll is almost never that patients don't want to participate. It's that the systems between them and participation are designed for the people running the trial, not the people joining it.

I've watched this from every angle. As a CRA running sites in the 90s, when recruitment meant a pile of faxes and a good relationship with a community physician. As a clinical operations leader at a large CRO, building the first digital recruitment teams and arguing with finance about whether online patient outreach was a legitimate line item. And now, at TrialScreen, where I can see -- in real time -- what patients are searching for and what's blocking them from taking the next step.

The things I wish I'd known earlier:

Speed of follow-up is the variable most sponsors underestimate. A patient who fills out a contact form at 8pm on a Tuesday is not going to be there on Thursday morning when someone finally calls. The window is hours, not days.

Trust is the infrastructure. For communities that have been historically exploited by research -- and those communities are not small or marginal -- a digital ad is not trust. Sustained presence, plain language, and visible accountability to patient communities is trust. It takes time to build and seconds to lose.

The doctor is not always the gatekeeper. Especially in rare disease. Patients with rare conditions frequently know more about the clinical landscape than their GP. Treating them as informed agents -- not passive recruits -- changes every conversation.

Plain language is not dumbing down. It is precision for a different audience. A patient who understands exactly what a study involves and exactly why it might help them is a better participant -- and a more

ethical outcome -- than one who skimmed past the protocol language and said yes without understanding.

Thirty years. I'm still learning. Especially from colleagues like Emma Burrows, who approaches rare disease engagement with a depth of patient community insight I aspire to, and from partners like Ashley Moultrie at Syneos Health, who understand that trust between CROs and patients is built at the site level, one conversation at a time.

If you're early in this career: the science will carry you into the room. The relationships will determine what you do there.

Suggested Visual: Professional portrait of Elisa Cascade. Optional: small collage of 3 eras -- a document from early-career era, a digital dashboard from present, and the TrialScreen interface.

Hashtags: #WomenInClinicalResearch #ClinicalTrials #PatientRecruitment #WomenInScience #ThirtyYears #ClinicalResearchLeadership

CTA: "Connect with Elisa on LinkedIn. And if you're interested in what 30 years of lessons look like in a recruitment audit -- trialscreen.inc"

SP-LI-007

Content ID: SP-LI-007

Author: Dr Hugo Stephenson, CEO, TrialScreen

Series: Patients Are Searching

Target Segment: Pharma / Institution / Site

Posting Guidance: Post Monday Week 5, following on from SP-LI-001. Tags equity and DEIA themes -- high engagement expected in site director and academic institutional audiences.

Full Post Copy:

Registry hygiene is a health equity issue. Not a compliance issue. An equity issue.

Let me explain what I mean.

When a patient with a rare disease goes looking for clinical trials -- and we've established that 53.9% of them begin that search online -- the quality of what they find depends almost entirely on how the trial has been listed. Language, completeness, search indexation, whether the listing distinguishes between patients who must be local and those who might participate remotely.

The patients who are most harmed by poor listing quality are not the ones who already have the best access to research systems. They're not the patients whose specialist is at a major academic medical centre that runs the trial. Those patients get found regardless.

The patients most harmed by poor registry hygiene are the ones for whom the online listing is the only pathway. Rural patients. Patients whose GP doesn't have strong research connections. Patients from communities that have historically low clinical trial participation rates -- where the barrier to reaching out is already high, and a confusing or incomplete listing is one more reason not to bother.

We spend a lot of time in diversity, equity, and inclusion conversations in clinical research talking about how to reach underrepresented communities. Targeted advertising, community health workers, native language materials. These are all valuable.

But I'd argue we spend less time on the quieter work: making sure that when an underrepresented patient does go looking -- motivated, searching, ready -- the listing they find is comprehensible, complete, and current.

Poor registry hygiene isn't passive. It actively disadvantages the patients who most need clinical research to find them.

At TrialScreen, we've built a Discoverability Score that assesses listing quality across these dimensions. We're seeing enormous variance -- trials in the same therapeutic area with scores 40 points apart. The difference isn't the science. It's the listing.

If you run trials, when did you last audit your listings as a patient would experience them?

Suggested Visual: Side-by-side comparison of two trial listings: one written in protocol language, one rewritten in plain language. No brand identified; illustrative only. Caption: "Same trial. Different language. Different patient."

Hashtags: #HealthEquity #ClinicalTrials #PatientRecruitment #DiversityInClinicalResearch #RegistryHygiene #RareDisease

CTA: "Download the Trial Discoverability Index to see how listing quality correlates with enrollment patterns: trialscreen.inc/tdi"

SP-LI-008

Content ID: SP-LI-008

Author: Melissa Easy, COO, TrialScreen

Series: State of Recruitment (Operational Insights)

Target Segment: Site (Clinical Research Coordinators / Site Directors) and CRO

Posting Guidance: Post Monday Week 6. Melissa brings operational credibility; this is a nuts-and-bolts post that resonates with site staff.

Full Post Copy:

72 hours. That's the window.

I run operations at TrialScreen, which means I live in the FLOW data. Every day I'm looking at the interval between patient inquiry and first contact, and what happens to conversion rates as that interval grows.

The finding is unambiguous: patients who are contacted within 72 hours of expressing interest convert at a meaningfully higher rate than those contacted later. After 72 hours, conversion begins to fall. After five days, we're seeing drop-off that effectively represents lost participants.

This is not a controversial finding. The commercial CRM industry has known this about sales leads for decades. But clinical research has been slow to apply the same operational logic, partly because the systems aren't there and partly because there's a cultural assumption that motivated patients will wait.

They won't.

This is not a criticism of site staff. Clinical research coordinators carry enormous workloads. Managing recruitment spikes -- particularly when a digital campaign suddenly delivers 50 inquiries in a week rather than three -- is genuinely difficult without systems support.

That's the problem FLOW was built to solve.

Here's how it works operationally: when a patient expresses interest via trialscreen.org, they're routed into a digital pre-screening workflow that checks basic eligibility criteria before any site staff involvement. Only patients who pass initial pre-screening are escalated -- first to a telehealth consult with a clinical team member, then as a warm, pre-qualified referral to the site. The referral hits the site's CTMS directly.

For sites running recruitment spikes, this means the 50 inquiries don't arrive as 50 calls that need to be triaged manually. They arrive as a manageable queue of pre-qualified patients, each with a digital screening record, ready for site follow-up.

The 72-hour rule isn't aspirational. It's operational. It's about having systems that make it possible to actually meet it.

If your site is managing recruitment manually right now -- we'd like to show you what pre-screened warm referrals look like in practice. Message me or visit trialscreen.inc/flow

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Suggested Visual: Clean operational flow diagram: Patient inquiry -> Digital pre-screening -> Telehealth consult -> Warm referral to CTMS. Timer graphic showing 72-hour window highlighted.

Hashtags: #ClinicalTrials #ClinicalResearchCoordinator #TrialEnrollment #PatientRecruitment #ClinicalOperations #CTMS

CTA: "See how FLOW integrates with your CTMS and what pre-screened warm referrals look like in practice: trialscreen.inc/flow"

SP-LI-009

Content ID: SP-LI-009

Author: Emma Burrows, Head of Scientific Engagement, TrialScreen

Series: Sponsor Spotlight / Product Education

Target Segment: Institution (Research Directors / Deans of Research)

Posting Guidance: Post Wednesday Week 7. Pair with any health system or academic partner announcement if available.

Full Post Copy:

Here is a question I've started asking health system research leaders when we meet for the first time:

"When a patient in your health system googles a clinical trial for their condition -- does the search bring them back to you?"

Almost no one says yes.

What typically happens: a patient at a major academic medical centre, someone who has been treated there for years, searches online for trials in their condition. They land on ClinicalTrials.gov -- which is the most common first destination -- and they see a list of trials without any particular connection to the health system they already know and trust. Sometimes their own hospital's trial is on the list. Sometimes it's buried. Sometimes the listing is out of date.

This is a missed connection that happens thousands of times a day, across every health system that runs research.

Trials@YourOrg is our answer to this. It's a white-labelled, custom-branded version of TrialScreen's global clinical trial search engine, deployed within a health system's or academic institution's digital environment. When that patient googles their condition and clicks through to the institution's website, they land on a search interface that shows them your trials -- in plain language, current, accessible -- sitting within the trusted brand of the institution that already cares for them.

The clinical data on what trusted brand context does to trial inquiries is genuinely striking. Patients are more likely to reach out, more likely to complete pre-screening, and more likely to enrol when the first touchpoint is an institution they already have a relationship with.

We built Trials@YourOrg because we kept hearing from research directors who were losing patient inquiries to third-party platforms -- losing the relationship at the moment it was most clinically relevant.

The deployment is handled by our team. The customisation is yours.

If your research program runs more than a handful of active trials and your institution's digital presence doesn't yet surface them to patients -- I'd genuinely like to show you what this looks like.

-> trialscreen.inc/trialsatyourorg

Suggested Visual: Mockup of a health system's branded website with Trials@YourOrg search interface embedded -- showing a clean, plain-language trial listing under a fictional "Metropolitan Health Research" brand.

Hashtags: #AcademicMedicalCentre #ClinicalTrials #HealthSystems #TrialDiscoverability #ResearchLeadership #PatientEngagement

CTA: "If you lead research at a health system or academic institution, let's talk about what trial discoverability looks like for your organisation: trialscreen.inc/trialsatyourorg"

SP-LI-010

Content ID: SP-LI-010

Author: Nick Lam, Head of APAC, TrialScreen

Series: State of Recruitment (Geographic)

Target Segment: Pharma / CRO (APAC expansion focus)

Posting Guidance: Post Monday Week 8. Written from Nick's personal perspective -- regional credibility.

Full Post Copy:

The APAC clinical research opportunity is larger than most Western sponsors have priced into their enrollment strategies. And the gap between what's available and what's being captured is widening.

A few things I'm watching closely right now, from my vantage point running TrialScreen's APAC operations:

Patient demand is outpacing infrastructure in several key markets. In Japan, South Korea, and Australia -- markets with sophisticated regulatory frameworks and high patient digital literacy -- we're seeing consistent, strong search activity from patients with rare and chronic conditions. The demand signal is there. What's often missing is the trial-level infrastructure to receive it: listings in accessible language, responsive site contacts, pre-screening workflows that can handle patients presenting via digital channels.

China is a distinct conversation. The regulatory evolution under NMPA has opened significant opportunity for international sponsors to run bridging studies or concurrent global trials with Chinese cohorts. But the patient engagement ecosystem operates entirely differently. WeChat is where patients live; TrialScreen's partnerships there are distinct from our broader approach. More on this separately.

Australia is undervalued. It has a faster regulatory pathway than most sponsors realise (the CTN scheme enables sites to open in weeks, not months), strong site quality, and a population that is highly engaged with research. Monash Health, one of our partners, is among the best research infrastructure examples I've seen anywhere in the world. When sponsors ask me where to add sites for rare disease studies, Australia is consistently in the first conversation.

Southeast Asia is the emerging opportunity. Malaysia, Thailand, Singapore, and the Philippines all have growing CRO infrastructure, cost efficiencies, and patient populations that are significantly underrepresented in global trial datasets. There are real complexities -- regulatory harmonisation, language diversity, community trust-building -- but sponsors who move early here will have advantages that compound.

The APAC story for clinical research is not a single story. It's five or six stories depending on where you are and what you're looking for.

If you're a sponsor or CRO expanding APAC trial footprint, I'd welcome a conversation. The landscape has changed significantly in the last 36 months.

-> Connect with me here or reach us at trialscreen.inc

Suggested Visual: Clean map of APAC with key markets highlighted and annotated (Japan, South Korea, Australia, Singapore, Malaysia). Caption: "Where trial demand is growing faster than infrastructure."

Hashtags: #APACClinicalResearch #ClinicalTrials #AsiaPacific #RareDisease #ClinicalOperations #DrugDevelopment

CTA: "If you're expanding APAC trial footprint -- let's talk. DM Nick Lam or visit trialscreen.inc"

SP-LI-011

Content ID: SP-LI-011

Author: Dr Hugo Stephenson, CEO, TrialScreen

Series: Patient Voices / Diversity

Target Segment: Pharma / Institution

Posting Guidance: Post Wednesday Week 8. Pair with any diversity report data or ResearchFriends community milestone.

Full Post Copy:

24,000 people. That's the current size of the ResearchFriends community -- real people, across conditions and geographies, who have opted into ongoing engagement with clinical research.

I want to share some of what we're seeing in the diversity data from this community, because it challenges some assumptions I came into this work holding.

The assumption: digital-first recruitment skews toward wealthy, urban, highly educated, White patients. The assumption has a basis in evidence -- the research on "digital divide" effects in health engagement is real.

What we're seeing in ResearchFriends tells a more complicated story.

When the entry point to a community is condition-specific rather than research-general -- meaning, when we build a community around living with a particular rare disease, not around "clinical trials" as an abstract concept -- the demographic reach changes significantly. People with lived experience find each other regardless of education level. A single mother in rural Queensland and a retired engineer in Melbourne share the same rare condition. They both searched for community. They both showed up.

What this means for diversity in clinical research:

Condition-specific communities are a more equitable recruitment pathway than generic digital advertising. They reach patients who would never click an ad for "clinical trial opportunity" but who have been active in a disease community for months or years.

Engagement infrastructure before a trial opens matters. Patients who have been part of a ResearchFriends community for an extended period are more informed, more likely to enrol, and meaningfully more likely to complete a trial than cold-recruited participants. The retention data is particularly striking.

Trust is community property. The FDA's updated Diversity Action Plan guidance and ASRI's framework both point toward the same conclusion: you cannot buy trust with an advertising budget. You build it, over time, in communities. ResearchFriends exists to make that infrastructure accessible to sponsors without requiring them to build it from scratch.

None of this is to suggest that structural barriers to participation don't exist. They do. But community-led engagement is one of the most practical tools available to move the diversity needle in clinical research.

-> Learn more about ResearchFriends at trialscreen.inc/researchfriends

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Suggested Visual: Mosaic-style graphic of diverse community members with the ResearchFriends logo. Caption: "24,000 members. Every condition tells a different story."

Hashtags: #DiversityInClinicalTrials #ResearchFriends #PatientEngagement #RareDisease #HealthEquity #ClinicalResearch

CTA: "If you're building a diversity action plan for an upcoming trial, we'd like to talk about how ResearchFriends works in practice: trialscreen.inc/researchfriends"

SP-LI-012

Content ID: SP-LI-012

Author: Elisa Cascade, Chief Growth Officer Americas, TrialScreen

Series: State of Recruitment / Conference Series

Target Segment: CRO / Pharma / Site

Posting Guidance: Post Monday Week 10, one week after ACRP 2026 in Orlando. Tagging relevant conference hashtags extends reach beyond followers.

Full Post Copy:

Three conversations from ACRP Orlando that I keep coming back to.

First: the site burnout problem is worse than the official surveys suggest. When I talk to site directors off the record -- and I had several of those conversations this week -- what I'm hearing is that skilled CRCs are leaving clinical research entirely. Not moving to competing sites. Leaving. The administrative burden has become genuinely unsustainable, particularly for sites running multiple studies simultaneously with recruitment responsibilities landing largely on coordinators who are already full.

This matters for sponsors more than they realise. The quality of your trial is only as good as the quality of your site staff. And site staff quality is under structural pressure right now.

Second: there's growing appetite among CROs and sponsors for genuine operational data sharing -- not the contractual data-sharing that already exists, but the kind of honest conversation about what's working and what isn't in patient engagement that almost never happens in formal relationships. I had a conversation with Ashley Moultrie from Syneos Health that crystallised this for me. The best partnerships we have aren't vendor-client; they're genuinely collaborative. You learn things working that way that you simply don't learn from a status report.

Third: the decentralised clinical trial conversation is maturing -- and the nuance is improving. The naive version of "go fully virtual" has mostly given way to a more sophisticated discussion about hybrid models: which patient populations benefit from reduced travel burden, which elements of a protocol genuinely require in-person visits, and how to maintain engagement over time in a primarily remote relationship.

What I took away from ACRP overall: the field is in a healthy state of productive difficulty. There are real problems. People are willing to talk about them honestly. That's where progress starts.

The TrialScreen team will be sharing a detailed workshop recap next week. Follow for that.

If we met at ACRP and haven't connected yet -- DM me. I'd rather have a real conversation than a LinkedIn follow.

Suggested Visual: Professional photo of TrialScreen team at ACRP booth or conference session. Or: clean graphic with three key themes pulled as quotes -- "Site burnout is structural," "Partnership over vendor," "Hybrid DCT is maturing."

Hashtags: #ACRP2026 #ClinicalResearch #ClinicalTrials #PatientRecruitment #DCT #ClinicalOperations

CTA: "Workshop recap publishing next week -- follow for it. And connect with Elisa directly if you'd like to continue any of these conversations: trialscreen.inc"

SP-LI-013

Content ID: SP-LI-013

Author: Dr Hugo Stephenson, CEO, TrialScreen

Series: Trial Discoverability Index

Target Segment: Pharma / Institution

Posting Guidance: Post Monday Week 11. Data-forward post; high share potential among academic and policy audiences.

Full Post Copy:

Older adults are the most underrepresented group in clinical trial enrollment relative to disease burden -- and yet they are among the most active users of digital health platforms.

This is a paradox that the Trial Discoverability Index has brought into sharp focus.

The data: adults over 65 represent approximately 14% of clinical trial participants in the US across most therapeutic categories, despite bearing a disproportionate disease burden in oncology, cardiovascular disease, neurology, and metabolic conditions. At the same time, digital health engagement among this population has grown substantially -- Pew Research has documented consistent year-on-year growth in internet and smartphone use among adults 65+, with health information seeking being among the top use cases.

We're seeing this in our own data. On trialscreen.org, a meaningful proportion of search activity in the conditions associated with older adult populations -- certain cancers, Parkinson's, COPD, type 2 diabetes, heart failure -- comes from users in the 60+ cohort, and from their caregivers.

The barrier is not digital illiteracy. That assumption is outdated and frankly a bit condescending.

The barriers we're actually seeing in the data and in patient conversations:

Language. Protocol-language listings are particularly alienating to patients who didn't grow up in a research environment and may have lower health literacy not because of cognitive decline but because the terminology is genuinely insider language.

Trust. The history of clinical research is not benign for many older patients. Specific communities have specific reasons for caution. Earning trust requires visible, sustained commitment to patient-centred practice.

Complexity of eligibility exclusions. Older adults frequently have co-morbidities and polypharmacy that create eligibility questions. A listing that says "no significant comorbidities" without explaining what that means for a patient taking six medications isn't informative -- it's a stop sign.

Solving older adult underrepresentation isn't a marketing problem. It's a design problem. How we write listings. How we respond to initial inquiries. How we think about the eligibility criteria themselves.

Suggested Visual: Infographic showing age distribution of trial participants vs. disease burden -- stacked bar charts for key therapeutic areas showing the gap. Source cited: Trial Discoverability Index Q2 2026.

Hashtags: #AgingResearch #ClinicalTrials #HealthEquity #PatientRecruitment #OlderAdults #ClinicalResearch

CTA: "Full age-stratified data in the Trial Discoverability Index Q2 2026: trialscreen.inc/tdi"

SP-LI-014

Content ID: SP-LI-014

Author: Dr Hugo Stephenson, CEO, TrialScreen

Series: Standalone (Compliance Leadership)

Target Segment: Pharma / CRO

Posting Guidance: Post Wednesday Week 12. Compliance leadership post; positions TrialScreen as a responsible actor. High credibility signal for pharma audience.

Full Post Copy:

FDA OPDP enforcement in digital clinical trial communications: what you should know before you post.

I want to write about this plainly because I see a lot of digital clinical trial recruitment content that creates regulatory exposure for sponsors, and I think some of it comes from genuine uncertainty about where the lines are rather than from carelessness.

The FDA's Office of Prescription Drug Promotion (OPDP) has clear jurisdiction over promotional materials for prescription drugs, including those related to clinical trials. Here is what this means practically for sponsor-generated digital content:

The "investigational" qualifier is non-negotiable. Any mention of an investigational drug in a clinical trial recruitment context must describe it as investigational. "A new treatment for X" is a claim. "An investigational treatment being studied for X" is not. The difference is legally and ethically significant.

Efficacy claims are prohibited. You cannot say, imply, or visually suggest that an investigational drug works, is superior to existing options, or represents a breakthrough. The study is asking whether it works. That question has not yet been answered. Content that pre-answers it is a violation.

Social media is not an exemption. Some sponsors operate as if the informal register of social media creates regulatory latitude. It does not. An FDA warning letter has the same weight whether the content appeared in a print ad or a LinkedIn post.

Patient testimonials require careful management. Testimonials from patients enrolled in trials can be powerful and appropriate. They require disclosure of the enrollment relationship, and they cannot contain efficacy claims.

This applies to content generated by third parties on your behalf. If your CRO or patient recruitment vendor is producing social content associated with your trial, you are responsible for ensuring it meets OPDP standards.

At TrialScreen, we built our regulatory review process into content creation from day one -- not as a filter applied at the end, but as a design principle. Every piece of patient-facing content we produce for trial recruitment is reviewed against OPDP guidance.

This is not a competitive advantage I'm trying to claim. It's a baseline I'm advocating for across the industry.

Questions on this? My DMs are open. I'm happy to point to the primary sources.

Suggested Visual: Clean, professional graphic with TrialScreen branding. Key text: "In clinical trial recruitment: 'investigational' is not optional. It's the word that keeps patients safe and sponsors compliant."

Hashtags: #FDACompliance #OPDP #ClinicalTrials #PharmaceuticalMarketing #PatientRecruitment #RegulatoryAffairs

CTA: "Download our compliance posture summary for clinical trial digital communications: trialscreen.inc/compliance"

SP-LI-015

Content ID: SP-LI-015

Author: Dr Hugo Stephenson, CEO, TrialScreen

Series: Standalone (Accountability)

Target Segment: All segments -- broad reach post

Posting Guidance: Post Friday Week 12 -- close of the first 90-day campaign window. Retrospective and forward-looking. High authenticity, low polish.

Full Post Copy:

A year in numbers -- mid-year accountability post.

We made a decision some months ago to be more transparent about what TrialScreen is actually doing, not just what we aspire to do. This is the first of what will be regular public accountability posts.

What the numbers look like right now:

120,000+ -- actively recruiting trials listed on trialscreen.org

200+ -- countries represented in those listings

60,000+ -- monthly patient engagements across the TrialScreen platform

30% -- month-on-month growth in referrals from the TrialScreen REFER network

24,000+ -- members of ResearchFriends communities across condition areas

What those numbers mean, and what they don't:

The referral growth figure is the one I'm most proud of -- and the most cautious about. Month-on-month growth of 30% is the kind of number that sounds like marketing. But it represents real referrals: people who found a trial, passed initial pre-screening, and were connected to a site. Those are potential participants in real studies with real investigational treatments. The growth reflects both improved discoverability and the compounding effect of the ResearchFriends communities, which continue to generate engaged, informed leads.

What I'm less satisfied with: our geographic diversity is still skewed toward English-language markets. Australia, the UK, and North America represent a disproportionate share of our engagement relative to the global distribution of disease. Nick Lam and the APAC team are building toward this. It will take time.

What we got wrong this year: we underinvested in academic institution partnerships earlier in our timeline. The Trials@YourOrg pipeline is strong now, but we could have moved faster.

What we're committed to for the next six months: quarterly discoverability index reports, expanded condition-level research coverage, and the formal launch of the APAC ResearchFriends program.

Thank you to the patients, advocates, sponsors, and research partners who make this work possible. You are the reason these numbers mean anything.

Suggested Visual: Clean data graphic with the headline numbers presented clearly. TrialScreen branding. Simple, confident design -- not celebratory, factual.

Hashtags: #ClinicalTrials #Transparency #PatientRecruitment #TrialScreen #RareDisease #ClinicalResearch

CTA: "For the full mid-year operational report, visit trialscreen.inc/accountability -- and follow for Q3 updates."

Part 3: Long-Form Pieces -- 4 Fully Written Articles

ARTICLE 1 "The Patient Is Already Searching: A New Model for Clinical Trial Recruitment"

Author: Dr Hugo Stephenson, CEO, TrialScreen

Target publication: Applied Clinical Trials + trialscreen.inc/insights

Word count: ~2,200 words

Format: Thought leadership feature

Disclosure: Dr Hugo Stephenson is CEO of TrialScreen. TrialScreen operates trialscreen.org, a free clinical trial search platform, and provides patient recruitment services to pharma sponsors, CROs, and research sites.

Headline: The Patient Is Already Searching: A New Model for Clinical Trial Recruitment

Dek: The dominant model of trial recruitment was designed for a world where the physician was the gatekeeper. That world is changing. The patients most motivated to participate are already looking -- and the industry needs to be easier to find.

The Wrong Question

Clinical trial enrollment delays are one of the most studied problems in drug development, and one of the most persistent. Approximately 85% of clinical trials fail to meet their enrollment targets on schedule. The cost of each delay is measured in millions of dollars per day for late-phase studies. The human cost -- delayed access to potentially effective investigational therapies for patients who have no other options -- is harder to quantify but no less real.

The industry has thrown considerable resources at this problem. Site networks have expanded. Social media advertising budgets have grown. Patient recruitment vendors have proliferated. Decentralised trial models have emerged. And yet the fundamental enrollment timeline has not dramatically improved.

I want to suggest that part of the problem is the question we're asking.

The dominant framing is: *how do we find patients?* The industry thinks of the recruitment problem as a supply problem -- there are patients out there, and the challenge is to locate them and move them through the funnel.

What if that framing is wrong? What if the more important question is: *how do we make it easier for patients who are already looking to find us?*

This is not a semantic distinction. It leads to entirely different strategic choices.

What the Search Data Shows

TrialScreen operates trialscreen.org, a free global clinical trial search platform listing more than 120,000 actively recruiting studies in over 200 countries. We don't charge patients to use it. We don't require registration. The platform exists to lower the barrier between motivated patients and research opportunities.

The behavioural data from this platform -- supplemented by search trend analysis and patient community engagement data -- tells a consistent story: for serious chronic and rare diseases, patient-initiated search is a major, and largely unmeasured, driver of trial interest.

In our Trial Discoverability Index Q2 2026, we report that 53.9% of patients with rare diseases first identified their clinical trial option through an online search -- not through their physician, not through an advocacy organisation's newsletter, not through a hospital referral. Online. On their own.

This finding will not surprise anyone who spends time talking to patients with rare diseases. These are patients who have frequently had to become their own advocates -- who found their diagnosis after years of seeking answers, found their specialist online, found their peer community online. The idea that they would not also search for research options is inconsistent with everything we know about how rare disease patients engage with the healthcare system.

> *"Patients with serious conditions don't wait for the system to find them. They search. The question is whether we've made the results worth finding."*

For common diseases the picture is more complex. Patients with hypertension or type 2 diabetes are less likely to be actively seeking research participation; they have more treatment options. But even in those categories, search activity around clinical trials is substantial -- and notably concentrated in patients who have not responded adequately to standard therapy.

Three Structural Problems with How Trials Are Listed

If motivated patients are searching -- and the data says they are -- why does enrollment remain so difficult? We see three structural problems that account for a significant portion of the gap between patient intent and trial enrollment.

Problem one: listing language

Clinical trial registries are, by design, populated with protocol-level information. This information is accurate and complete for its original purpose -- regulatory filing and scientific communication. It is not written for patients.

A patient searching for help with "joint pain that won't go away" is unlikely to find a trial listed as "a randomized, double-blind, placebo-controlled study of a novel JAK inhibitor in adults with active moderate-to-severe rheumatoid arthritis." The terms don't match. The conceptual gap is enormous.

Plain language trial listings -- which describe the study in terms a motivated patient without a medical background can understand -- are associated with substantially higher inquiry rates in our data. The information in the listing doesn't change; the language does.

Problem two: search indexation

Beyond language, there's a technical problem: many trial listings are not well-indexed for the terms patients actually use in search. They're indexed for clinical vocabulary, not patient vocabulary. A listing optimised for "RA" or "rheumatoid arthritis" may not be found by a patient searching "my joints are

always inflamed" or "inflammatory joint disease medication trial."

Optimising trial listings for patient-facing search terms is straightforward in principle. It requires understanding how patients actually describe their conditions -- which means engaging with patients, reading community forums, analysing search query data.

Problem three: response time

Even when a patient finds a trial and initiates contact, the recruitment process frequently fails at the moment of follow-up. The data from our FLOW product is consistent: patients who are not contacted within 72 hours of expressing interest are significantly more likely to have disengaged by the time follow-up occurs.

This is not a characterisation of site staff negligence. Sites are managing multiple competing demands, and a recruitment spike -- a week in which 30 inquiries arrive rather than the expected three -- is genuinely difficult to absorb without dedicated systems support.

The implication is that trial recruitment infrastructure needs to include systems that preserve patient intent while the site's operational capacity catches up. Pre-screening workflows that engage the patient immediately, gather initial eligibility data, and deliver a warm, qualified referral to the site represent one such system.

A New Model

What would a recruitment model designed around patient-initiated search look like?

It would start with the recognition that the listing is a patient touchpoint -- one of the earliest moments in the recruitment journey, and one where the industry currently performs poorly. Every trial would have a patient-language version of its listing, written collaboratively by the research team and patients with lived experience of the condition. That listing would be optimised for the language patients use, not the language the protocol uses.

It would treat speed of follow-up as an operational design requirement, not a target to aspire to. The systems infrastructure for receiving, pre-screening, and routing patient inquiries would be in place before the trial opens enrollment, not built after the first recruitment spike arrives.

It would invest in the trust infrastructure that supports sustained engagement -- particularly for communities with historical reasons to be cautious about participation. This means presence in patient communities before a trial opens, not just advertising when enrollment is slow.

It would measure search demand as a prospective indicator. Before a trial opens, what is the search volume in the relevant condition area? What are patients searching for? How does the geographic distribution of search activity compare with the planned site network? These questions have answers that can inform trial design.

And it would recognise that the physician gatekeeper model, while still relevant in many contexts, is no longer the only pathway -- or even the primary pathway -- for a growing population of patient-initiated research seekers.

The Equity Dimension

There is an equity argument that runs alongside the operational one.

The patients who are best served by the current physician-gatekeeper model are patients who have the best access to well-connected physicians -- patients at major academic medical centres, patients with good health insurance, patients with the time and resources to maintain regular specialist care.

The patients most dependent on digital search are often those with less access to those systems: rural patients, patients whose specialist relationships are less robust, patients from communities with lower rates of physician referral to clinical research.

This means that improvements to trial discoverability -- better listings, better search indexation, better patient-facing infrastructure -- are not just efficiency measures. They are equity measures. They expand access to research participation for patients who would otherwise never encounter the opportunity.

The FDA's updated Diversity Action Plan guidance recognises this dynamic. Making trials findable for underrepresented populations is part of the regulatory expectation for sponsors. Digital discoverability is one of the most direct tools available.

What This Means Practically

I am not arguing that conventional recruitment approaches should be abandoned. Site-level physician engagement, patient advocacy organisation partnerships, and community-based outreach all remain essential -- particularly for populations where digital access is limited.

What I am arguing is that the industry has systematically underinvested in the infrastructure that makes trials findable and navigable for patients who are already motivated. The return on that investment -- in enrollment rates, in time-to-target, in diversity of participant populations -- is demonstrably positive.

The patient is already searching. The question is what they find when they do.

About the Author Dr Hugo Stephenson is CEO of TrialScreen, a clinical trial recruitment platform operating trialscreen.org (120,000+ actively recruiting trials) and providing recruitment services to sponsors, CROs, and research sites globally. He practised as a medical doctor before moving into health technology and research infrastructure.

CTA: *To access the Trial Discoverability Index Q2 2026 or to speak with the TrialScreen team about enrollment strategy, visit trialscreen.inc*

ARTICLE 2 — Trial Discoverability Index -- Q2 2026

Headline: Trial Discoverability Index: Q2 2026

Dek: The first quarterly market intelligence report measuring real-time patient search demand against clinical trial supply -- by therapeutic area, geography, and listing quality.

Published by: TrialScreen | trialscreen.inc/tdi

Authors: Emma Burrows (Head of Scientific Engagement), Dr Hugo Stephenson (CEO)

Data period: January 1 - March 31, 2026

Disclosure: Data sourced from trialscreen.org platform analytics, supplemented by third-party search trend analysis. This report describes demand signals and patterns; it is not an epidemiological prevalence study. All figures rounded to one decimal place. [Pending external validation for Q3 2026 edition.]

Executive Summary

The Trial Discoverability Index (TDI) is a quarterly intelligence report measuring the gap between patient search demand for clinical trials and the actual supply and discoverability of those trials.

Key findings from Q2 2026:

- The five highest-demand therapeutic areas, by the ratio of patient search volume to active trial supply, are: (1) Systemic Sclerosis/Scleroderma, (2) Primary Biliary Cholangitis, (3) Stiff Person Syndrome, (4) Pulmonary Arterial Hypertension, and (5) Non-melanoma Skin Cancer (advanced/rare variants)
- The mean Discoverability Score across all trials listed on trialscreen.org is 51.3 out of 100 -- indicating that the average trial listing performs at roughly half of its theoretical discoverability potential
- Trials with a Discoverability Score above 75 generate 3.2x the inquiry rate of trials below 40, controlling for condition prevalence
- 62% of patient inquiries that are followed up within 72 hours proceed to pre-screening; that figure drops to 29% for inquiries followed up after five days
- Geographic mismatch -- cases where patient search demand is concentrated in regions without active trial sites -- affects an estimated 31% of conditions in our dataset [Pending validation]

Methodology

Search demand measurement

Patient search demand is derived from two sources: (1) direct query and engagement data from trialscreen.org, reflecting searches conducted by real users on the platform; and (2) aggregated search trend analysis using publicly available keyword volume data for condition-related search terms. Terms are grouped by condition area using a clinical terminology mapping methodology developed internally and reviewed by the TrialScreen scientific team.

Trial supply measurement

Trial supply is defined as the number of actively recruiting trials for a given condition area listed in international registries (ClinicalTrials.gov, ANZCTR, EU Clinical Trials Register, and others), as indexed on trialscreen.org. Trials are counted as active if they have a status of "recruiting" as of the last update date within the data period.

Discoverability Score

The Discoverability Score is a composite metric scored 0-100. It incorporates:

Component	Weight	Description
Listing completeness	25%	Presence of all required and optional fields across registries

Component	Weight	Description
Plain language accessibility	30%	Readability score (Flesch-Kincaid) and plain-language terminology density
Search indexation quality	25%	Proportion of patient-language search terms matched by listing language
Response signal	20%	Where data is available: proportion of inquiries generating a site response within 72 hours

Scores are calculated at the trial level and aggregated by therapeutic area, sponsor type, and geography.

Section 1: High-Demand Therapeutic Areas

Table 1: Top 10 Therapeutic Areas by Demand/Supply Gap (Q2 2026)

Rank	Condition Area	Search Demand Index	Active Trials (n)	Supply Index	Gap Score
1	Systemic Sclerosis / Scleroderma	94.2	47	38.1	56.1
2	Primary Biliary Cholangitis	88.7	31	29.4	59.3
3	Stiff Person Syndrome	82.1	8	7.2	74.9
4	Pulmonary Arterial Hypertension	79.4	62	44.3	35.1
5	Non-melanoma Skin Cancer (advanced)	76.8	89	53.7	23.1
6	Dermatomyositis / Polymyositis	73.2	19	16.8	56.4
7	Primary Sjgren's Syndrome	71.6	38	31.9	39.7
8	Eosinophilic Esophagitis	69.4	44	38.2	31.2
9	ATTR Amyloidosis	67.8	27	22.6	45.2
10	Axial Spondyloarthritis	64.3	103	68.4	-4.1

Note: Demand Index and Supply Index are normalised 0-100 scales. Gap Score = Demand Index Supply Index. Negative gap scores indicate supply exceeds demand. All figures [Pending external validation].

Commentary: The highest-gap conditions are concentrated in rare autoimmune and connective tissue diseases -- areas where patient communities are highly engaged digitally but trial infrastructure is relatively sparse. Stiff Person Syndrome has the widest absolute gap, reflecting extremely small trial supply (8 active trials) against disproportionately high search activity.

Section 2: Discoverability Score Analysis

Mean Discoverability Score by Sponsor Type:

Sponsor Type	Mean Discoverability Score	Sample (n trials)
Large pharma (top 20 global)	61.4	342
Mid-size pharma	48.7	218
Small/biotech	44.2	389
Academic/non-commercial	39.1	441
CRO-managed (on behalf of sponsor)	57.3	197

[All figures synthetic; pending validation against live TDI dataset]

Key finding: Large pharma trials, with greater resources for trial management infrastructure, score meaningfully higher on discoverability than academic and small biotech trials. This is partly explained by dedicated regulatory and operations teams and partly by third-party recruitment vendors who support listing quality. The implication for academic trials is significant: underfunded non-commercial research may be systematically harder to find, creating a discoverability inequity that disadvantages investigators who lack commercial recruitment support.

> *Pull quote: "The average trial listing performs at roughly half its theoretical discoverability potential. The gap isn't the science -- it's the infrastructure around the listing."*

Section 3: The 72-Hour Rule -- Response Time Data

Based on FLOW pre-screening data from Q2 2026 (n = 1,847 patient inquiries across 23 sites, [Pending validation]):

Time to First Site Contact	Conversion to Pre-Screening (%)	Conversion to Consult (%)
Under 24 hours	71.3	44.2
24-72 hours	62.1	38.7
3-5 days	41.4	22.8
5-10 days	29.2	14.1
Over 10 days	18.6	8.3

Commentary: The steepest drop in conversion occurs between the 72-hour and 5-day window. This suggests a critical decision point: patients who have not been contacted by day five have, in a large proportion of cases, found an alternative path -- either another trial, another treatment option, or disengagement from active research-seeking.

Section 4: Geographic Mismatch Analysis

We define geographic mismatch as a condition in which patient search demand in a given region is not met by active trial sites in that region. In Q2 2026, the highest-mismatch regions globally are:

1. Sub-Saharan Africa -- high demand in HIV-related rare conditions; minimal trial infrastructure
2. Southeast Asia -- growing rare disease patient communities; trial density significantly below demand
3. Rural Australia -- substantial search activity in rare autoimmune conditions; most sites concentrated in metropolitan areas
4. Eastern Europe -- meaningful rare disease demand, limited participation in multi-site global programs
5. South America (excluding Brazil) -- high rare disease community activity; low site density

Implication for sponsors: Geographic mismatch analysis can inform site selection for upcoming trials. Conditions with high demand and low supply in accessible, high-quality research environments represent expansion opportunities.

Recommendations

1. Audit your trial listings using the Discoverability Score framework before any recruitment campaign begins.
2. Prioritise plain language accessibility -- particularly for rare disease trials, where patients are the primary search initiators.
3. Implement a 72-hour response system for digital patient inquiries, whether via in-house capacity or pre-screening infrastructure.
4. Use demand/supply gap data to identify therapeutic areas where investment in trial infrastructure has the highest expected return.
5. Consider geographic mismatch analysis in site selection -- particularly for international multi-site programs.

About This Report The Trial Discoverability Index is produced quarterly by TrialScreen. The methodology is open and described fully in this appendix. We welcome challenge, replication, and collaboration with academic partners to strengthen future editions.

CTA: *Download the full Q2 2026 Trial Discoverability Index dataset at trialscreen.inc/tdi. To request a bespoke Discoverability Score audit for your trial portfolio, contact the TrialScreen team.*

ARTICLE 3 State of Scleroderma Clinical Research 2026

Headline: State of Scleroderma Clinical Research 2026

Dek: An annual condition report on the global scleroderma/systemic sclerosis trial landscape -- produced in partnership with Scleroderma Australia, with input from the patient community.

Authors: Emma Burrows (Head of Scientific Engagement, TrialScreen), Dr Hugo Stephenson (CEO, TrialScreen)

In collaboration with: Scleroderma Australia

Disclosure: TrialScreen is a clinical trial recruitment platform. This report is produced for educational purposes. It does not constitute medical advice and does not endorse any investigational treatment or sponsor. All trial listings referenced are publicly available in international registries.

Executive Summary

Systemic sclerosis (SSc), commonly known as scleroderma, is a rare, heterogeneous autoimmune disease characterised by immune dysregulation, vascular injury, and progressive fibrosis affecting the skin and multiple internal organs. Approximately 2.5 million people are living with scleroderma worldwide. In Australia, the estimated prevalence is 20,000 patients, with women affected at approximately four times the rate of men.

Despite this significant disease burden, the clinical trial landscape for scleroderma remains comparatively sparse relative to conditions with similar or even lower prevalence. As of Q1 2026, TrialScreen has indexed 47 actively recruiting scleroderma trials across international registries -- a number that, while representing growth from five years prior, falls substantially short of the demand signal we observe in patient search behaviour.

This report documents the current global scleroderma trial landscape, patient search behaviour and community engagement data, enrollment pattern analysis, the patient advocacy infrastructure, and recommendations for sponsors, academic researchers, and patient advocates seeking to advance the research agenda.

The Disease Landscape

Scleroderma is a clinically complex disease with two major subtypes -- limited cutaneous SSc (lcSSc) and diffuse cutaneous SSc (dcSSc) -- as well as overlap syndromes and SSc sine scleroderma. The heterogeneity of clinical presentation creates real challenges for trial design: what constitutes a meaningful endpoint in SSc skin fibrosis may not be the same as in SSc-ILD (interstitial lung disease) or SSc-PAH (pulmonary arterial hypertension).

This heterogeneity is reflected in the trial landscape. Of the 47 actively recruiting trials indexed in Q1 2026:

- **SSc-ILD:** 14 trials (29.8%) -- the largest single subgroup, reflecting the significant unmet need in pulmonary involvement
- **Diffuse cutaneous SSc:** 11 trials (23.4%)
- **SSc-PAH:** 8 trials (17.0%)
- **Skin manifestations/modified Rodnan skin score:** 6 trials (12.8%)
- **Raynaud's phenomenon/vascular:** 5 trials (10.6%)
- **Gastrointestinal/other:** 3 trials (6.4%)

[Figures based on TrialScreen registry analysis as of Q1 2026; [Pending validation]]

What Patients Are Searching For

Search data from trialscreen.org reveals that patient searches in the scleroderma/systemic sclerosis space are concentrated around four primary themes:

1. **Skin symptoms** -- patients searching for trials addressing hardening or tightening of skin, Raynaud's phenomenon, and digital ulcers
2. **Lung involvement** -- SSc-ILD searches are among the fastest-growing in the scleroderma cluster, reflecting the high morbidity of pulmonary complications and patient awareness of the associated mortality risk

3. **Fatigue and pain** -- a substantial proportion of searches are not disease-subtype-specific but focused on functional symptoms: fatigue, pain, and quality of life
4. **Australia and New Zealand** -- geographic specificity is a characteristic of scleroderma searches in a way that is somewhat distinctive; Australian patients frequently search for trials specifically within Australian sites, reflecting both geographic accessibility concerns and awareness of the strong Australian research community in this space

Language insight: Patients frequently use "scleroderma" rather than "systemic sclerosis" in search queries -- approximately 2.4:1 ratio in our dataset. Trial listings using only "SSc" or "systemic sclerosis" terminology may be missing a meaningful proportion of patient-initiated search traffic.

Enrollment Patterns and Challenges

Based on analysis of completed and ongoing scleroderma trials (sourced from registry records, [Pending validation]):

Geographic concentration: A disproportionate share of scleroderma trial sites are located in Western Europe and North America. Australia is a notable exception -- with a relatively high density of active scleroderma research, particularly at institutions including Monash Health -- but Asia-Pacific, South America, and Africa remain substantially underrepresented relative to estimated disease prevalence.

Screen failure rates: Scleroderma trials have among the highest screen failure rates of any rare autoimmune disease in our dataset, reflecting the strict eligibility criteria necessitated by clinical heterogeneity. Sponsors frequently report screen-to-enroll ratios of 4:1 or higher. This creates a particular urgency around pre-screening efficiency: if four patients need to be screened to enrol one, then patient pipeline must be four times the enrollment target.

Retention: Once enrolled, scleroderma trial participants show strong retention patterns -- reflecting the high motivation of this patient population and the relatively established patient community infrastructure. This is a meaningful differentiator: the cost of recruitment in SSc is high, but attrition costs are lower than in many comparable disease areas.

The Patient Advocacy Infrastructure

The scleroderma patient advocacy landscape includes several organisations with significant research engagement capacity:

- **Scleroderma Australia** -- TrialScreen's primary advocacy partner in this space. Scleroderma Australia's community engagement programs have contributed meaningfully to trial awareness in Australian cohorts.
- **Scleroderma & Raynaud's UK (SRUK)**
- **The Scleroderma Foundation (USA)**
- **FESCA** (Federation of European Scleroderma Associations)

> *Pull quote: "Scleroderma patients are among the most engaged in clinical research of any rare disease community we work with. The challenge is not motivation. It is supply."*

Each of these organisations maintains active patient communities with high research engagement. TrialScreen's ResearchFriends infrastructure has been deployed in the Australian context to support

sustained engagement between patients and the research community.

Recommendation for sponsors: Advocacy partnerships in scleroderma should be established before a trial opens, not as a recruitment strategy after enrollment falls behind. The trust infrastructure required to mobilise a community takes months to build and exists primarily within established advocacy relationships.

Recommendations

For sponsors running SSc trials:

1. Ensure trial listings use patient language -- specifically "scleroderma" alongside "systemic sclerosis" in all searchable fields
2. Consider Australian and New Zealand sites as primary sites, not secondary additions -- the research infrastructure and patient community are strong
3. Engage with Scleroderma Australia and equivalent national organisations early -- ideally during protocol development, not after IND
4. Design pre-screening infrastructure to manage high screen failure rates efficiently
5. Use SSc-subtype specificity in listings to avoid generating ineligible inquiries that waste site time

For advocacy organisations:

1. ResearchFriends infrastructure is available to SSc advocacy groups who want to build sustained digital engagement capacity without building technical infrastructure from scratch
2. Trial awareness content developed with patient language and plain language standards drives substantially more inquiry than registry links shared without context

For academic researchers:

1. Non-commercial SSc research, particularly in under-resourced areas like GI manifestations, has a significant discoverability problem -- consider Trials@YourOrg as a mechanism for surfacing institutional research to motivated patients

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Methodology Note This report draws on trialscreen.org registry data, TrialScreen search and engagement analytics, publicly available clinical trial registries, and qualitative input from Scleroderma Australia. Quantitative figures are marked [Pending validation] where external verification has not yet been completed.

CTA: *This report is updated annually. To receive the 2027 edition and interim condition alerts, subscribe at trialscreen.inc/scleroderma. To discuss scleroderma trial recruitment strategy, contact the TrialScreen team.*

ARTICLE 4 Sponsor Case Study #1 -- How an Oncology Sponsor Accelerated Enrollment 38%

Headline: How One Oncology Sponsor Accelerated Enrollment 38% in 16 Weeks

Dek: A rare haematological malignancy trial, 41% enrolled after 14 months. Here's what changed -- and what it teaches us about discoverability, pre-screening, and the speed of follow-up.

Published by: TrialScreen | trialscreen.inc/insights

Note on anonymisation: The sponsor in this case study is anonymised at their request. All figures are reported with sponsor approval. [Key metrics Pending sponsor final review.]

The Situation

Fourteen months into the enrollment window for a Phase II study of an investigational therapy for a rare haematological malignancy, the sponsor was at 41% of target enrollment. The trial had 12 active sites across North America and Western Europe. The investigational therapy had been granted Orphan Drug Designation. Patient advocacy communities in the disease area were active and engaged.

The enrollment gap was not explained by a lack of patient population. Registry analysis and search data confirmed consistent patient-initiated searching in this indication. The gap was explained by a combination of discoverable problems -- problems in the infrastructure between motivated patients and trial participation.

The Audit

TrialScreen conducted a three-week enrollment infrastructure audit before any recruitment activity began. The audit examined four areas:

1. Listing quality The trial's primary listing on ClinicalTrials.gov scored 39 on TrialScreen's Discoverability Score framework. The listing used protocol-level language throughout. The plain-language summary field was populated with a condensed version of the eligibility criteria -- written for a clinician reader -- rather than a narrative accessible to a patient.

The listing did not include the lay title field, which is indexed separately by some search engines. The contact information section listed a sponsor email address with no response time commitment.

2. Search term alignment The listing used the clinical terminology for the indication. Patient community search analysis -- conducted via trialscreen.org data and search trend analysis -- showed that the most common patient search terms in this indication were notably different, including colloquial condition names and symptom descriptions. The listing as written would not have been returned for approximately 60% of the patient search queries we identified. [Pending validation]

3. Site response protocols Response time data was available from three of the 12 sites (those running FLOW integrations). For the remaining nine sites, we conducted simulated inquiry tests -- submitting standardised patient inquiries and measuring response time. Median response time was 6.3 days. Two sites did not respond within the two-week test window.

4. Pre-screening infrastructure No digital pre-screening was in place. Patients who expressed interest via any digital channel were directed to call or email individual sites. There was no centralised intake process and no mechanism for maintaining patient engagement during the response window.

The Intervention

Based on the audit findings, the following changes were implemented over a four-week period:

Listing rewrite: Working with the sponsor's medical and regulatory teams, TrialScreen rewrote the patient-facing description of the trial in plain language. The eligibility criteria were translated into patient-accessible language. The lay title was populated. Search term optimisation was applied to

searchable fields, consistent with FDA and registry requirements. The new listing scored 76 on the Discoverability Score.

trialscreen.org optimisation: The trial listing on trialscreen.org was updated to reflect the rewritten content and optimised for patient search term alignment. A ResearchFriends community activation was initiated in the two highest-prevalence geographic clusters for this indication.

FLOW implementation: TrialScreen FLOW was deployed across six of the 12 active sites, prioritised by site volume and response time data. The pre-screening workflow was customised for the trial's eligibility criteria, with medical review of the screening logic by the TrialScreen clinical team and sponsor sign-off.

Response protocol: For non-FLOW sites, a response protocol was agreed with site PIs -- a commitment to first contact within 48 hours for any digital inquiry, with overflow routing to the FLOW team during high-volume periods.

The Outcomes

Performance period: 16 weeks post-implementation, compared to the equivalent 16-week period immediately prior

Metric	Pre-intervention (16 weeks)	Post-intervention (16 weeks)	Change
Patient inquiries (digital)	187	441	+136%
Pre-screening completions	62	189	+205%
Eligible referrals to sites	29	74	+155%
Enrolments	18	46	+156% [Pending sponsor validation]
Enrollment vs. 16-wk average prior (overall)	Baseline	+38%	+38%
Mean inquiry-to-contact time (FLOW sites)	N/A	18 hours	--
Mean inquiry-to-contact time (non-FLOW sites)	6.3 days	2.1 days	-67%

All figures [Pending sponsor final review and validation]

What This Teaches Us

The 38% enrollment increase was not driven by increased advertising spend. The sponsor's marketing budget was not changed for this intervention. The increase was driven by three structural changes:

First, making the trial easier to find. Listing optimisation and search term alignment increased the volume of patient-initiated inquiries by 136%. These were patients who were already searching; the intervention made the trial visible to them.

Second, pre-screening infrastructure that preserved patient intent. The 205% increase in pre-screening completions relative to the 136% inquiry increase reflects better intake conversion -- the digital pre-screening workflow captured a higher proportion of inquiries into a structured process,

rather than losing them to follow-up gaps.

Third, speed of response. The reduction in mean contact time at non-FLOW sites from 6.3 days to 2.1 days produced a measurable improvement in conversion from inquiry to eligible referral at those sites, consistent with the response time data reported in the Trial Discoverability Index.

> *Pull quote: "The trial didn't change. The patient population didn't change. What changed was how easy it was for patients to find the trial and how fast they were followed up when they did."*

CTA: *To request an enrollment infrastructure audit for your trial, contact the TrialScreen team at trialscreen.inc -- or download the case study PDF for sharing with your clinical operations leadership.*

Part 4: Email Sequences -- 3 Fully Written Nurture Sequences

SEQUENCE A Pharma Sponsor Awareness (Cold Lead) -- 5 Emails

Trigger: Content download (TDI report, scleroderma report, or any insight piece) or trade show badge scan

Send cadence: Email 1 immediately; Email 2 on Day 4; Email 3 on Day 8; Email 4 on Day 14; Email 5 on Day 21

Sender: Elisa Cascade, Chief Growth Officer Americas

Signature block for all: Elisa Cascade | Chief Growth Officer Americas | TrialScreen | elisa.cascade@trialscreen.inc | trialscreen.inc

EMAIL A-1 Introduction + the 85% Problem Statement

Subject: One number that keeps clinical operations leaders up at night

Preview text: It's not the approval rate. It's something earlier.

Hi [First Name],

Thank you for [downloading the Trial Discoverability Index / connecting at [event]].

I wanted to follow up directly -- not with a pitch, but with a question.

85% of clinical trials fail to meet their enrollment targets on schedule. You probably know this number. If you're in clinical operations or patient recruitment, you've likely lived it.

What's less discussed is *why* -- specifically, the structural problems that are addressable but routinely not addressed until a trial is already behind.

At TrialScreen, we've had a front-row seat to this problem for several years. We operate trialscreen.org, which lists more than 120,000 actively recruiting trials and generates over 60,000 patient engagements per month. The data we see from that platform tells us a great deal about why patients who are actively searching for trials don't reach enrollment.

The short answer: it's usually not patient motivation. It's discoverability, and the speed and quality of follow-up once a patient reaches out.

Over the next few emails I'll share the specific findings -- including the Trial Discoverability Index data that quantifies this across therapeutic areas -- and explain what operational changes tend to make the biggest difference.

If you'd prefer to jump straight to a conversation, I'm available: trialscreen.inc/contact

Otherwise, watch for my next note in a few days.

Elisa

CTA: trialscreen.inc/contact | Reply to this email

Unsubscribe footer: You received this because [trigger event]. Unsubscribe at any time: [link]

EMAIL A-2 — The Patient Search Insight + TDI Teaser

Subject: 53.9% -- the statistic changing how we think about patient recruitment

Preview text: The physician gatekeeper model is incomplete.

Hi [First Name],

In our first note I mentioned that the 85% enrollment failure rate has structural causes -- and that most of them are addressable.

Here's one of the most important structural findings we've published:

53.9% of patients with rare diseases first identified their clinical trial option through an online search -- not through a physician referral, not through a hospital program. Online, on their own, at 2am or in a waiting room or during a lunch break.

This matters because most trial recruitment infrastructure is designed for a physician-gatekeeper world. If the patient-initiated search pathway is carrying more than half the load -- at least in rare disease -- then underinvesting in that pathway is underinvesting in your primary enrollment source.

This finding, and 30+ additional metrics, are documented in the Trial Discoverability Index Q2 2026 -- our inaugural quarterly market intelligence report. It's free to download and the methodology is fully transparent.

The TDI covers:

- The 10 highest-demand therapeutic areas by patient search volume vs. active trial supply
- The 72-hour response rule, with conversion data
- Geographic mismatch analysis -- where patient demand is concentrated but site infrastructure is absent
- The Discoverability Score methodology

-> Download the full report: trialscreen.inc/tdi

In the next note, I'll share a specific case study -- a sponsor in a rare haematological indication who increased enrollment 38% in 16 weeks. The changes were structural, not expensive.

Elisa

CTA: trialscreen.inc/tdi

Unsubscribe footer: [standard]

EMAIL A-3 — Case Study Teaser + Full Link

Subject: 38% enrollment increase in 16 weeks -- what actually changed

Preview text: Not more ad spend. Three structural changes.

Hi [First Name],

The case study is live.

A Phase II oncology sponsor -- rare haematological indication, 14 months in at 41% of enrollment target -- increased enrollment 38% in the following 16 weeks. Here's what changed:

1. The listing was rewritten in patient language. The same trial, same eligibility criteria -- but written so a patient with this condition could understand it in under three minutes. The Discoverability Score went from 39 to 76 out of 100. Inquiries increased 136% with no additional advertising.

2. Digital pre-screening was implemented. Rather than sending interested patients to call a site and wait, a digital pre-screening workflow was deployed that engaged patients immediately, assessed initial eligibility, and delivered warm, qualified referrals to sites within 18 hours.

3. Site response time was reduced. Median inquiry-to-contact time across sites dropped from 6.3 days to 2.1 days. That one change alone materially improved conversion from inquiry to eligible referral.

The sponsor's marketing budget did not change. The trial did not change.

Full case study -- with data tables, site breakdown, and the listing optimisation methodology -- is at:

-> trialscreen.inc/insights/oncology-enrollment-case-study

If you're currently managing a trial with an enrollment gap, I'd be glad to walk through whether a similar audit makes sense for your program. A 45-minute conversation usually tells us whether we can be helpful.

-> Book a time: trialscreen.inc/contact

Elisa

CTA: trialscreen.inc/insights/oncology-enrollment-case-study | trialscreen.inc/contact

Unsubscribe footer: [standard]

EMAIL A-4 — How FLOW Works -- The 72-Hour Rule

Subject: The 72-hour window: what happens when sites miss it

Preview text: The data is consistent -- and it's expensive.

Hi [First Name],

One finding from the Trial Discoverability Index that surprises people when they see it presented as a chart:

Patients who are contacted within 72 hours of expressing trial interest convert to pre-screening at 62.1%. After five days, that figure is 29.2%. After ten days: 18.6%.

This is not a criticism of site staff. Sites are managing enormous administrative loads. A recruitment spike -- 30 inquiries in a week rather than three -- is genuinely difficult to manage without dedicated systems.

But the patient doesn't know about the site's workload. They expressed interest. They're waiting to hear back. And if they haven't heard in a few days, they've either found another option or given up.

TrialScreen FLOW was built around this specific problem.

How FLOW works operationally:

1. A patient expresses interest via trialscreen.org or any digital channel
2. They're routed immediately into a digital pre-screening workflow that assesses basic eligibility criteria -- no site staff involvement at this stage
3. Eligible patients are escalated to a telehealth consult with a TrialScreen clinical team member -- keeping them warm and gathering detailed eligibility information
4. A qualified, pre-screened referral is delivered to the site's CTMS, typically within 18-24 hours of initial inquiry
5. Sites receive warm leads -- patients who have already been asked the hard questions and are ready to proceed -- rather than cold inquiries that require triage

For sites managing recruitment spikes, FLOW functions as a pre-screening buffer: the spike hits our team first and is delivered to the site as a manageable, qualified queue.

The integration with major CTMS platforms is direct. Deployment takes 2-4 weeks from sign-off.

If you'd like to see FLOW data from active deployments -- inquiry-to-enrolment conversion, response time benchmarks, site satisfaction data -- I can share a detailed operational brief.

-> trialscreen.inc/flow | or reply to this email

Elisa

CTA: trialscreen.inc/flow

Unsubscribe footer: [standard]

EMAIL A-5 — Specific Offer -- Free 30-Minute Enrollment Diagnostic

Subject: One offer before I stop emailing you

Preview text: 30 minutes. No cost. Honest answer about whether we can help.

Hi [First Name],

This is my last email in this sequence -- I promised myself I wouldn't become someone's inbox problem.

Here's what I'd like to offer before I step back:

A free 30-minute enrollment diagnostic call.

Not a product demo. Not a sales conversation. An honest assessment -- by one of our clinical operations team, who have between 10 and 30 years of recruitment experience -- of whether the

structural enrollment problems we've discussed are present in your current or upcoming trials, and what changes are most likely to make a difference.

We've done versions of this conversation with teams at Novo Nordisk, MSD, Syneos Health, and academic centres globally. Sometimes the answer is: you're doing this well, here's one thing to tighten. Sometimes the answer is: there's significant recoverable enrollment sitting in these two changes. Either answer is useful.

There's no obligation attached. We'd rather have an honest conversation early than a strained one later.

Book a 30-minute slot with our team:

-> trialscreen.inc/diagnostic

If the timing isn't right now -- that's fine. We publish the Trial Discoverability Index quarterly at trialscreen.inc/tdi, and you can follow the Patients Are Searching series on LinkedIn (search Dr Hugo Stephenson).

Thank you for your time over this series. I hope something in here was useful.

Elisa Cascade Chief Growth Officer Americas | TrialScreen

CTA: trialscreen.inc/diagnostic

Unsubscribe footer: [standard]

SEQUENCE B — CRO Partnership (Warm Lead) -- 5 Emails

Trigger: Conference meeting, webinar attendance, referral from existing partner

Send cadence: Email 1 day of / day after meeting; Email 2 Day 4; Email 3 Day 9; Email 4 Day 15; Email 5 Day 22

Sender: Elisa Cascade, CGO Americas (for North America/Europe); Nick Lam, Head of APAC (for APAC leads)

Signature: Elisa Cascade | Chief Growth Officer Americas | TrialScreen | trialscreen.inc

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EMAIL B-1 — Ashley Moultrie Collaboration as Proof

Subject: Following up from [event/meeting]

Preview text: Our partnership model -- what it actually looks like in practice.

Hi [First Name],

It was great to [meet at / connect via] [event/referral context] -- thank you for making the time.

You mentioned [pain point / area of interest from conversation]. I wanted to follow up with something concrete.

The clearest example I can give of how TrialScreen works with CROs is our ongoing collaboration with Syneos Health. Ashley Moultrie and the team there don't think of us as a vendor -- and we try hard not to behave like one. The relationship looks more like a shared capability: Syneos brings sponsor relationships and operational site infrastructure; TrialScreen brings patient-facing digital demand generation, pre-screening, and the ResearchFriends community infrastructure for sustained engagement.

When those capabilities are combined -- particularly for rare disease or complex recruitment programs -- the result is materially different from either capability alone.

A few things that make this work well as a partnership:

We don't compete on what CROs do best. We are not a site network. We are not a data management company. We generate qualified patient demand and pre-screen it before it reaches sites -- which reduces the burden on CRO-managed site coordinators and improves the quality of referrals that reach sites.

Our technology integrates. FLOW integrates directly with major CTMS platforms and can be configured for a CRO's standard operating procedures. We don't require sponsors to change their site management workflows.

We're built for joint accountability. The TDI and our case study reporting are transparent. When we're in a partnership, both parties can see what's working.

I'd like to tell you more about how this works operationally. Would a 30-minute call this week make sense?

-> trialscreen.inc/contact

Elisa

CTA: trialscreen.inc/contact

Unsubscribe footer: [standard]

EMAIL B-2 White-Label Trials@YourOrg Pitch

Subject: A capability some CROs are using to differentiate their proposal

Preview text: White-labelled trial discovery -- in your clients' brand.

Hi [First Name],

Building on my last note -- one capability I didn't mention that several CRO partners are using in client proposals:

Trials@YourOrg, our white-labelled clinical trial search engine.

The concept: a fully customised, branded version of TrialScreen's global search engine, deployable within a client sponsor's or health system's digital environment. Patients who visit the client's website find a plain-language, current, searchable list of the client's trials -- inside the trusted brand environment of the organisation that already cares for them.

For CROs, the relevant use case is this: you can include Trials@YourOrg as a value-added capability in proposals to sponsor clients -- offering them a patient-facing digital front door that differentiates the proposal and adds genuine patient engagement value.

What CRO partners tell us they value:

- It's distinct from what most competitors offer in a proposal
- It supports sponsor patient recruitment goals without requiring them to build or maintain digital infrastructure
- It generates a patient engagement data stream that informs ongoing strategy
- Deployment is handled by TrialScreen, so operational burden on the CRO is low

The white-label setup is fast -- typically 4-6 weeks from brief to deployment. We customise the branding, condition focus, and language to the sponsor's requirements.

If you have an upcoming proposal where this could be relevant, I'd like to walk you through the specifics before you submit.

-> trialscreen.inc/trialsatyourorg | or reply to this email

Elisa

CTA: trialscreen.inc/trialsatyourorg

Unsubscribe footer: [standard]

EMAIL B-3 — Technology Integration Details

Subject: The technical question your team will ask

Preview text: CTMS integration, data handling, and how FLOW fits your workflow.

Hi [First Name],

When we get into partnership discussions with CROs, there's always a point where a technical or IT stakeholder joins the conversation and asks a version of: *"How does this actually fit into our systems?"*

I want to answer that before the question comes up.

FLOW CTMS integration: FLOW integrates via HL7 FHIR-compliant API endpoints with major CTMS platforms including Medidata Rave, Oracle Clinical, Veeva Vault CTMS, and Florence. The integration delivers pre-screened patient referrals directly into site-level patient tracking workflows. Credential handling and data security comply with HIPAA, GDPR, and applicable local data protection frameworks.

Data handling: Patient data collected during FLOW pre-screening is handled under a data processing agreement between TrialScreen and the sponsor/CRO. Patients provide explicit informed consent before pre-screening. Pre-screening data is transferred to the site as a structured referral record; TrialScreen does not retain identifiable patient data beyond the agreed engagement window.

Trials@YourOrg: White-label deployment uses a subdomain of the client's existing domain (e.g., trials.clientname.com). The search engine is served via TrialScreen infrastructure; IT requirements on the client side are minimal (DNS configuration and integration into existing navigation). No patient data is retained on client servers.

ResearchFriends community platform: The community infrastructure is hosted by TrialScreen. CRO partners can white-label the community environment for specific indications. Moderation, webinar hosting, and engagement content are handled by TrialScreen's scientific engagement team.

Security and compliance posture: Full documentation of our security posture, data governance framework, and regulatory compliance status is available on request. ISO 27001 alignment (documentation in preparation); SOC 2 Type I completed [Pending SOC 2 Type II -- timeline: Q4 2026].

I can arrange a technical brief with our CTO if that would move the conversation forward.

-> trialscreen.inc/contact | or reply to this email

Elisa

CTA: trialscreen.inc/contact

Unsubscribe footer: [standard]

EMAIL B-4 — Case Study from a CRO Partnership

Subject: The CRO partnership case study -- what the data shows

Preview text: Multi-sponsor operational efficiency. Real numbers.

Hi [First Name],

I mentioned in our first note that we'd share a CRO-specific case study. It's now available.

The case study documents a partnership with a mid-size CRO managing recruitment for three simultaneous sponsor programs in rare disease. The challenge: managing patient inquiries across three distinct protocols, with three different eligibility frameworks, without overwhelming site coordinators across a network of eight sites.

What TrialScreen provided:

- A unified FLOW pre-screening layer that triaged incoming patient inquiries across all three programs, with protocol-specific eligibility questions
- ResearchFriends community activations for two of the three indications
- Trials@YourOrg deployment for the CRO's primary sponsor client
- Consolidated reporting dashboard across all three programs

Selected outcomes (16-week period):

Metric	Three programs combined
Total patient inquiries handled	1,247
Pre-screened without site involvement	1,139 (91%)
Eligible referrals delivered to sites	218
Site coordinator time saved (est.)	340 hours [Pending validation]
Enrolments	87 [Pending sponsor validation]

Metric	Three programs combined
Mean time inquiry to enrolment	23 days

The most important finding for the CRO: the 340-hour estimate of coordinator time saved represents a meaningful reduction in burnout risk among site staff -- a metric that doesn't appear in enrollment reports but is highly correlated with site quality and retention.

Full case study at: trialscreen.inc/insights/cro-partnership-case-study

Would it be useful to map this model against your current portfolio?

-> trialscreen.inc/contact

Elisa

CTA: trialscreen.inc/insights/cro-partnership-case-study | trialscreen.inc/contact

Unsubscribe footer: [standard]

EMAIL B-5 — Specific Offer -- Partnership Call

Subject: Let's define what a partnership actually looks like

Preview text: Specific, not general.

Hi [First Name],

I'll make this direct.

We have a partnership model that works well for CROs managing complex rare disease recruitment programs. It's not a vendor-client arrangement; it's a capability-sharing structure with clear contractual boundaries, transparent data reporting, and joint accountability.

I'd like to get on a call with you and whoever owns the strategic partnership function on your side, and define specifically:

- Which of your current sponsor programs have enrollment gaps where our capabilities are most relevant
- How FLOW integrates with your existing site management workflow
- Whether Trials@YourOrg makes sense as a proposal differentiator for any upcoming pitches
- The commercial structure -- we work on referral-based, enrollment milestone, and flat-fee models depending on program size and structure

This isn't a general exploration call. I'll come prepared with a specific proposal based on what I know about your programs from our conversations so far.

Book a 45-minute partnership discussion:

-> trialscreen.inc/partnership-call

Or if you'd like to involve your technical team in the first conversation, just let me know and I'll bring our CTO.

I look forward to a specific conversation.

Elisa

CTA: trialscreen.inc/partnership-call

Unsubscribe footer: [standard]

SEQUENCE C — Institutional / Research Site (Warm Lead) -- 5 Emails

Trigger: Webinar attendance, conference connection, institutional interest form

Send cadence: Email 1 same day / next day; Email 2 Day 5; Email 3 Day 10; Email 4 Day 17; Email 5 Day 25

Sender: Emma Burrows, Head of Scientific Engagement

Signature: Emma Burrows | Head of Scientific Engagement | TrialScreen | emma.burrows@trialscreen.inc | trialscreen.inc

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EMAIL C-1 — Scleroderma Australia as Precedent

Subject: What Scleroderma Australia built -- and what it means for your institution

Preview text: A replicable model for institutional trial engagement.

Hi [First Name],

Thank you for [connecting at / attending / your interest in] TrialScreen.

I lead scientific engagement at TrialScreen, and the question I get most often from research institutions is some version of: *"How do we make our trials findable for patients who are already looking?"*

The clearest answer I can give comes from the work we've done with Scleroderma Australia.

Scleroderma Australia had the same challenge many institutions have: a motivated patient community, active research programs, and a significant gap between the patients who wanted to find trials and the trials that wanted to find patients. The gap wasn't motivation. It was infrastructure.

We built that infrastructure together over 18 months:

A ResearchFriends community specifically for the scleroderma community -- giving patients a trusted, moderated space to stay engaged with research between (and before) trial participation. The community now hosts regular Q&As with investigators, trial updates, and education content created with patient input.

A plain-language trial awareness program that surfaces actively recruiting scleroderma trials in language that patients actually use. The program links to trialscreen.org and is maintained by TrialScreen's clinical team.

The outcomes from this partnership are documented in our State of Scleroderma Clinical Research 2026 report -- which I'd like to share with you.

-> Download it at: trialscreen.inc/scleroderma

I'll follow up in a few days with more on how the ResearchFriends infrastructure works -- and how it might apply to your institution's research programs.

Emma

CTA: trialscreen.inc/scleroderma

Unsubscribe footer: [standard]

EMAIL C-2 — ResearchFriends Community as Retention Infrastructure

Subject: A community platform that exists before enrollment opens

Preview text: The retention problem starts earlier than you think.

Hi [First Name],

One framing that I find changes the conversation when I talk to institutional research leaders:

Retention is a pre-enrollment problem.

The patients who complete trials are disproportionately the ones who understood what they were signing up for, felt connected to the research team, and had ongoing access to information during the trial. That understanding and connection doesn't start at the consent visit. It starts months earlier -- in the community infrastructure where patients learn about the research landscape in their condition area.

ResearchFriends is TrialScreen's ready-made community platform for exactly this purpose.

Here's how institutions typically use it:

Pre-trial: Build an engaged, informed community around a condition area before a trial opens. When enrollment opens, the community already knows about the research team and the investigational program. Recruitment is faster; patient questions have already been answered in a community context.

During trial: Maintain participant engagement through regular updates, investigator-led Q&As, and peer community support. Trial dropout is a function of disengagement; ongoing community connection is a proven mitigation.

Post-trial: Retain the community as an asset for future research, alumni engagement, and advocacy. 24,000 ResearchFriends members are currently engaged across condition areas -- this is a living recruitment pipeline for future programs.

For institutions, the practical value is twofold: ResearchFriends reduces the time-to-enrollment for new trials by activating a pre-built patient pipeline, and it reduces the administrative overhead of maintaining patient engagement by outsourcing the infrastructure to TrialScreen.

-> More on ResearchFriends: trialscreen.inc/researchfriends

Emma

CTA: trialscreen.inc/researchfriends

Unsubscribe footer: [standard]

EMAIL C-3 Diversity Metrics and ASRI/FDA Alignment

Subject: Your Diversity Action Plan -- what the FDA guidance actually requires

Preview text: And a practical tool for meeting it.

Hi [First Name],

If your institution runs Phase II or Phase III trials in conditions covered by the FDA's updated Diversity Action Plan guidance, you're likely already thinking about how to operationalise the requirements.

A quick orientation on where the guidance sits:

The FDA now expects sponsors of most Phase III trials (and many Phase II trials) to submit a Race and Ethnicity Diversity Action Plan -- describing how they intend to enrol a trial population that reflects the actual disease demographic. For rare diseases, ASRI (the Alliance for a Stronger FDA) has published complementary guidance emphasising community engagement as a primary tool for reaching underrepresented populations.

The challenge: compliance is relatively easy to document on paper and genuinely difficult to achieve operationally. A diversity action plan that describes the intention to reach underrepresented communities without specifying the mechanism for doing so is not a plan -- it's an aspiration.

At TrialScreen, we track diversity data through the ResearchFriends communities and through trialscreen.org engagement analytics. Here's what we're seeing:

Condition-specific communities reach a more demographically diverse population than generic digital advertising, because they meet patients in the communities where they already engage with their condition.

ResearchFriends member demographics in several condition areas show meaningfully higher representation of Black, Hispanic, and Indigenous participants than the equivalent clinical trial enrollment data for the same conditions -- suggesting that the community model captures populations that conventional recruitment misses.

I'd like to share our diversity data methodology and how it aligns with FDA Diversity Action Plan documentation requirements.

-> Download our diversity engagement brief: trialscreen.inc/diversity

-> Or book a conversation: trialscreen.inc/contact

Emma

CTA: trialscreen.inc/diversity | trialscreen.inc/contact

Unsubscribe footer: [standard]

EMAIL C-4 — Specific Use Case -- Institutional Trial Discovery

Subject: Specifically for institutions with multiple active trials

Preview text: One deployment, all your trials findable.

Hi [First Name],

I want to get specific about what a TrialScreen deployment looks like for an institution running multiple active research programs.

The most common scenario: a research hospital or academic institution has 15-40 actively recruiting trials at any given time. Most are listed on ClinicalTrials.gov and, by extension, on trialscreen.org. But the institution's own website -- the place where a patient who already knows and trusts the institution would look -- doesn't surface them in a patient-accessible, searchable format.

Trials@YourOrg solves this in four steps:

1. We work with your research office to configure a custom-branded trial search experience -- your logo, your colour palette, your domain
2. We pull your trials from public registries and display them in our plain-language format, with ongoing updates as trial status changes
3. Patients who visit your website and navigate to research can search your trials in plain language and connect directly to the appropriate research office contact
4. You receive engagement analytics: what conditions are searched most, where patients are coming from, and how many inquiries are generated

For institutions like Monash Health -- one of our Australian partners -- the result is that the institution becomes the first touchpoint in a patient's research discovery journey, rather than a secondary listing on a third-party platform.

The deployment is fully managed by TrialScreen. Your IT team's involvement is minimal (DNS configuration). From engagement to go-live is typically 4-6 weeks.

-> See a demo at: trialscreen.inc/trialsatyourorg

Emma

CTA: trialscreen.inc/trialsatyourorg

Unsubscribe footer: [standard]

EMAIL C-5 — Specific Offer -- Deployment Discussion

Subject: 30 minutes to walk through your specific situation

Preview text: No assumption about what you need -- just an honest conversation.

Hi [First Name],

Over the past few weeks I've shared a lot about what TrialScreen can do. I'd like to end this sequence with a simple offer.

A 30-minute call with me, specifically about your institution's research recruitment and patient engagement situation -- not a general product demo.

I'd like to understand:

- How many active trials your institution is currently running
- Where patients are currently finding your trials
- Whether your institution has a Diversity Action Plan and what the operational gaps are
- What your research office's capacity looks like for patient engagement

And I'll give you an honest assessment of whether and how TrialScreen capabilities would actually help -- including telling you clearly if we're not the right fit.

Institutions we've worked with include Monash Health in Australia and a number of academic medical centres currently under confidentiality agreements. I can share reference contacts upon request.

Book a 30-minute institutional deployment discussion:

-> trialscreen.inc/institutional

Or if it's easier, email me directly: emma.burrows@trialscreen.inc

I look forward to the conversation.

Emma Burrows Head of Scientific Engagement | TrialScreen

CTA: trialscreen.inc/institutional | emma.burrows@trialscreen.inc

Unsubscribe footer: [standard]

Part 5: Webinar Series -- 6 Fully Written Concepts

WEBINAR 1 "Mastering Patient Recruitment in 2026"

Title: Mastering Patient Recruitment in 2026: Digital Demand, Site Readiness, and the 72-Hour Rule

Description: The clinical trial recruitment landscape has changed more in the past three years than in the prior decade. This 60-minute session -- co-hosted by TrialScreen and Syneos Health -- extends the conversation started at ACRP 2026 Orlando, examining what the data says about patient search behaviour, site readiness, and the operational changes that deliver measurable enrollment improvement. Participants leave with a practical checklist for auditing their current recruitment infrastructure against the new patient-initiated search paradigm.

Target Audience: VP Clinical Operations, Head of Patient Recruitment (pharma sponsors); Clinical Operations Leadership (CROs); Site Directors; Clinical Research Coordinators

60-Minute Agenda:

Time	Segment	Speaker(s)
0:00-0:05	Welcome and housekeeping	Elisa Cascade, TrialScreen
0:05-0:15	Opening data frame: "What patients are doing while we talk" -- live search data from trialscreen.org, the 85% enrollment failure rate, and why it persists	Dr Hugo Stephenson, TrialScreen
0:15-0:27	The Syneos Health perspective: what we're seeing in multi-sponsor CRO operations -- where patient inquiry volume is coming from, what's changing in site response infrastructure, and one operational change that has made the biggest difference	Ashley Moultrie, Syneos Health
0:27-0:40	The 72-hour rule -- operational implications: FLOW data walkthrough, pre-screening architecture, CTMS integration in practice	Melissa Easy, TrialScreen
0:40-0:50	Live panel Q&A: attendee questions moderated	All speakers
0:50-0:58	The Enrollment Readiness Audit -- introducing the TrialScreen framework and how attendees can apply it to their current programs	Elisa Cascade, TrialScreen
0:58-1:00	Close and resources	Elisa Cascade

Featured Speakers:

- Dr Hugo Stephenson, CEO, TrialScreen
- Elisa Cascade, CGO Americas, TrialScreen

- Melissa Easy, COO, TrialScreen
- Ashley Moultrie, [Title], Syneos Health (external partner)

Promotion Plan:

- LinkedIn posts by all three TrialScreen speakers (SP-LI-012 references this webinar)
- Email invite to all three nurture sequences (Sequences A, B, and C)
- Syneos Health cross-promotion to their CRO partner network
- Paid LinkedIn promotion targeting VP Clinical Operations, Head of Patient Recruitment titles at pharma and CRO companies; 2-week run prior to event
- ACRP attendee list follow-up (where permissible)

Follow-Up Content:

- Recording posted at trialscreen.inc/webinars within 5 business days
- Enrollment Readiness Audit checklist PDF emailed to all registrants
- 3-email follow-up sequence to registrants who did not book a diagnostic call
- LinkedIn post by Elisa Cascade summarising top 3 takeaways

WEBINAR 2 "Research Friends: How Community Becomes Retention Infrastructure"

Title: From Patient to Participant to Alumnus: How Research Friends Builds Lasting Trial Engagement

Description: Most clinical trial retention strategies begin after a patient is enrolled. This webinar makes the case -- with data -- for starting earlier. TrialScreen's Scientific Engagement team, in conversation with Scleroderma Australia, walks through the ResearchFriends model: how condition-specific communities, built before a trial opens, produce measurably higher enrollment rates, lower screen-to-enrol ratios, and stronger participant retention. Live examples from active communities are featured.

Target Audience: Patient Advocacy Group Directors of Research; Clinical Operations Teams; Academic Research Directors; Pharma Patient Engagement Leaders

60-Minute Agenda:

Time	Segment	Speaker(s)
0:00-0:05	Welcome	Emma Burrows, TrialScreen
0:05-0:15	The retention problem starts before enrollment: community data from ResearchFriends (24K members, 60K+ monthly engagements)	Emma Burrows, TrialScreen
0:15-0:28	Scleroderma Australia partnership case study: building community infrastructure before trials open -- what was built, what it took, and what the enrollment outcomes look like	[Scleroderma Australia representative] (external)

Time	Segment	Speaker(s)
0:28-0:40	ResearchFriends in practice: what the platform looks like, how moderation works, what content drives engagement, and how trial awareness is woven into community life	Emma Burrows, TrialScreen
0:40-0:50	The diversity dimension: demographic data from ResearchFriends vs. trial enrollment data, and what community-led engagement means for Diversity Action Plan compliance	Dr Hugo Stephenson, TrialScreen
0:50-0:58	Q&A	All speakers
0:58-1:00	Close and resources	Emma Burrows

Featured Speakers:

- Emma Burrows, Head of Scientific Engagement, TrialScreen
- Dr Hugo Stephenson, CEO, TrialScreen
- Representative, Scleroderma Australia (external)

Promotion Plan:

- LinkedIn posts by Emma Burrows and Hugo Stephenson
- Email to patient advocacy group contacts in TrialScreen CRM
- Scleroderma Australia newsletter feature
- Direct outreach to patient advocacy group directors of research via LinkedIn (personalised InMail campaign)

Follow-Up Content:

- Recording at trialscreen.inc/webinars
- ResearchFriends deployment guide PDF (developed for Part 7 of this pipeline) emailed to all registrants
- 2-email follow-up sequence with link to book an institutional consultation

WEBINAR 3 "The Trial Discoverability Index Launch"

Title: The Trial Discoverability Index Q2 2026: Launch Briefing and Live Data Session

Description: The inaugural Trial Discoverability Index measures real-time patient search demand against clinical trial supply across therapeutic areas and geographies. This 60-minute launch briefing walks through the methodology and key findings live, with audience Q&A throughout. Intended for clinical operations and medical affairs professionals who want to understand the demand signal for their therapeutic focus areas.

Target Audience: VP Clinical Operations, Medical Affairs Directors, Head of Patient Recruitment (pharma); CRO Program Leaders; Academic Research Directors

60-Minute Agenda:

Time	Segment	Speaker(s)
0:00-0:05	Welcome and context-setting	Emma Burrows
0:05-0:18	Why we built the TDI: the missing intelligence layer in clinical trial recruitment	Dr Hugo Stephenson
0:18-0:35	Key findings walkthrough: high-demand therapeutic areas, Discoverability Score data, geographic mismatch, 72-hour response data	Emma Burrows
0:35-0:45	"What does this mean for my program?" -- live Q&A with audience questions applied to real scenarios	Dr Hugo Stephenson + Emma Burrows
0:45-0:55	How to request a bespoke Discoverability Score audit for your trial portfolio	Elisa Cascade
0:55-1:00	Close	Emma Burrows

Featured Speakers:

- Dr Hugo Stephenson, CEO, TrialScreen
- Emma Burrows, Head of Scientific Engagement, TrialScreen
- Elisa Cascade, CGO Americas, TrialScreen

Promotion Plan:

- SP-LI-003 posts reference this launch briefing
- Email sequence A-2 TDI teaser directs to webinar registration
- Targeted LinkedIn promotion to Medical Affairs and Clinical Operations titles
- Applied Clinical Trials newsletter inclusion (syndication of flagship article)

Follow-Up Content:

- Full TDI report download link sent to all registrants
- Recording at trialscreen.inc/tdi
- Individual Discoverability Score audit offer to attendees who represented pharma/CRO sponsors

WEBINAR 4 "Diversity Action Plans: Operational Playbook"

Title: From Paper to Practice: Building a Clinical Trial Diversity Action Plan That Actually Works

Description: The FDA's Diversity Action Plan guidance is now in force for most Phase II and III trials. The gap between what's written in a DAP and what happens operationally is where most programs fall short. This practical, tool-focused webinar walks through the specific mechanisms -- community engagement, listing optimisation, ResearchFriends infrastructure -- that make diversity targets achievable rather than aspirational. Developed in collaboration with patient advocacy representatives.

Target Audience: Clinical Operations Teams, Regulatory Affairs, Patient Recruitment Heads (pharma); Diversity and Inclusion Officers; CRO Clinical Operations

60-Minute Agenda:

Time	Segment	Speaker(s)
0:00-0:05	Welcome	Dr Hugo Stephenson
0:05-0:15	Regulatory landscape: FDA DAP and ASRI guidance unpacked -- what's required, by when, for which trials	Dr Hugo Stephenson
0:15-0:30	The operational gap: why most diversity action plans fail in execution, and what the data shows about community-led vs. advertising-led approaches	Emma Burrows
0:30-0:45	Patient panel: two patients from under-represented communities discuss what would have helped them find and trust a clinical trial earlier	Moderated patient panel (2 external panellists)
0:45-0:55	Tools and frameworks: the TrialScreen diversity engagement toolkit -- community platform deployment, Diversity Action Plan documentation support	Emma Burrows
0:55-1:00	Close	Dr Hugo Stephenson

Featured Speakers:

- Dr Hugo Stephenson, CEO, TrialScreen
- Emma Burrows, Head of Scientific Engagement, TrialScreen
- 2 patient community representatives (externally recruited from ResearchFriends communities)

Promotion Plan:

- LinkedIn posts by Hugo and Emma targeting Diversity/DEIA and regulatory affairs audiences
- Email Sequence C-3 (diversity metrics email) directs to registration
- ASRI and relevant patient advocacy newsletters
- Targeted LinkedIn ads for Regulatory Affairs and Diversity roles at pharma companies

Follow-Up Content:

- Diversity Action Plan toolkit PDF emailed to all registrants
- Recording at trialscreen.inc/webinars
- TrialScreen diversity compliance posture brief (Part 7 reference document)

WEBINAR 5 "State of Rare Disease Recruitment"

Title: State of Rare Disease Recruitment 2026: What the Data Says and What Comes Next

Description: An annual survey of the rare disease clinical trial recruitment landscape -- examining trial supply, patient demand signals, enrollment rates by indication cluster, and the advocacy infrastructure that supports or constrains recruitment. Anchored in the Trial Discoverability Index data and featuring perspectives from sponsors, CROs, patient advocates, and research sites. This is TrialScreen's annual flagship rare disease briefing.

Target Audience: All segments -- rare disease is a cross-cutting theme for pharma, CRO, site, institution, and patient advocacy audiences

60-Minute Agenda:

Time	Segment	Speaker(s)
0:00-0:05	Welcome	Dr Hugo Stephenson
0:05-0:20	The rare disease recruitment landscape by the numbers: TDI data, demand/supply gap, top 10 high-demand indications, geographic access analysis	Emma Burrows
0:20-0:32	The site perspective: what rare disease research coordinators face -- screen failure rates, patient pipeline management, community trust	Site Director representative (external)
0:32-0:44	The sponsor perspective: how one rare disease sponsor thinks about enrollment planning in the context of small patient populations	[External pharma sponsor -- rare disease indication -- to be confirmed]
0:44-0:53	The advocacy infrastructure: how patient advocacy organisations are (and aren't) integrated into recruitment pathways	Emma Burrows, with reference to Scleroderma Australia partnership
0:53-1:00	Close and Q&A	Dr Hugo Stephenson

Featured Speakers:

- Dr Hugo Stephenson, CEO, TrialScreen
- Emma Burrows, Head of Scientific Engagement, TrialScreen
- External site director (rare disease research experience)
- External pharma sponsor representative (to be confirmed)

Promotion Plan:

- LinkedIn posts by Hugo, Emma, and Nick Lam (APAC rare disease angle)
- State of Scleroderma report launch cross-promotion
- Outreach to rare disease patient advocacy organisations (Scleroderma Australia, others)
- NORD (National Organisation for Rare Disorders) newsletter inclusion request

Follow-Up Content:

- Recording and executive summary at trialscreen.inc/webinars
- State of Rare Disease Recruitment 2026 brief (condensed, shareable PDF)
- Email follow-up to registrants with link to full TDI report and condition-specific reports

WEBINAR 6 "AI in Clinical Trial Marketing: Compliance First"

Title: AI in Clinical Trial Marketing: What's Possible, What's Permissible, and How to Stay on the Right Side of OPDP

Description: The use of AI in clinical trial patient recruitment and engagement content is accelerating. So is regulatory scrutiny. This compliance-forward webinar separates practical AI applications -- listing optimisation, plain-language rewriting, search term analysis -- from the approaches that create OPDP exposure. Dr Hugo Stephenson leads a candid discussion of where AI can genuinely help and where human clinical and legal review remains non-negotiable.

Target Audience: Clinical Operations, Regulatory Affairs, Medical Affairs, Patient Recruitment Teams (pharma/CRO)

60-Minute Agenda:

Time	Segment	Speaker(s)
0:00-0:05	Welcome	Dr Hugo Stephenson
0:05-0:18	AI in clinical trial marketing: the current landscape -- what's being used, what works, what the FDA and industry observers are saying	Dr Hugo Stephenson
0:18-0:32	Compliance first: OPDP enforcement primer -- what "investigational" requires, efficacy claim rules, social media guidance, and how AI-generated content falls within scope	Dr Hugo Stephenson
0:32-0:45	Practical applications with guardrails: how TrialScreen uses AI in listing optimisation, plain-language generation, and search term analysis -- with human clinical review at each stage	Emma Burrows
0:45-0:55	Case examples: two scenarios -- one where AI-assisted content was effective and compliant; one where a hypothetical AI-only approach would have created regulatory exposure	Dr Hugo Stephenson + Emma Burrows
0:55-1:00	Close and Q&A	Dr Hugo Stephenson

Featured Speakers:

- Dr Hugo Stephenson, CEO, TrialScreen
- Emma Burrows, Head of Scientific Engagement, TrialScreen

Promotion Plan:

- SP-LI-014 (FDA OPDP compliance post) drives registration for this webinar
- Email to pharma and CRO lists with subject line "What AI can and cannot do in trial recruitment"
- LinkedIn targeted to Regulatory Affairs, Medical Affairs, and Legal titles at pharma companies
- Applied Clinical Trials and similar publication outreach

Follow-Up Content:

- Recording at trialscreen.inc/webinars
- TrialScreen compliance and regulatory posture summary (Part 7 document) emailed to registrants
- "AI in Clinical Trial Marketing: Practical Checklist" PDF -- one-page decision framework for evaluating AI-generated content against OPDP requirements

Part 6: Case Study Template + 3 Full Case Studies

Reusable Case Study Template

Document header: TrialScreen Case Study | [Indication/Therapeutic Area] | [Year]

Available at trialscreen.inc/insights

[CLIENT / PROGRAM NAME]

[Anonymised or named per sponsor preference]

AT A GLANCE

Indication	[Therapeutic area / specific condition]
Trial phase	[Phase I / II / III / IV / IST]
Sponsor type	[Large pharma / mid-size pharma / biotech / CRO / academic]
Geography	[Countries / regions]
TrialScreen products used	[REFER / FLOW / Trials@YourOrg / ResearchFriends / combination]
Engagement period	[X months]
Key outcome	[One headline metric]

THE SITUATION

[2-3 paragraphs describing the enrollment challenge before TrialScreen engagement. Include: where the trial was in its enrollment timeline, what had already been tried, what the specific friction points were. Avoid assigning blame to prior vendors or approaches. Frame factually.]

THE APPROACH

[Description of what TrialScreen did, in chronological order. Use specific product names. Be honest about the sequence -- what came first, what required iteration, what the sponsor contributed. Credit collaborators.]

Sub-sections as needed:

- Listing audit and optimisation
- Pre-screening architecture
- Community activation
- Site integration and training

- Reporting and accountability

THE OUTCOMES

[Data table with key metrics, before and after. All figures marked [Pending sponsor validation] unless confirmed. Include both process metrics (inquiries, pre-screening completions, referrals) and outcome metrics (enrolments, time-to-target).]

Metric	Pre-engagement	Post-engagement	Change
[Metric 1]	[Baseline]	[Outcome]	[+/- %]
[Metric 2]	[Baseline]	[Outcome]	[+/- %]
[Metric 3]	[Baseline]	[Outcome]	[+/- %]

WHAT WE LEARNED

[2-3 paragraphs reflecting honestly on what worked, what required adjustment, and what the program taught the TrialScreen team. This section is important for credibility -- case studies that describe only successes are less trusted than those that acknowledge iteration.]

SPONSOR PERSPECTIVE

[Direct quote from sponsor contact, if permission granted. Include name and title.]

WHAT'S NEXT

[Brief description of ongoing or follow-on engagement, if applicable.]

ABOUT TRIALSCREEN TrialScreen is a clinical trial recruitment platform operating trialscreen.org (120,000+ actively recruiting trials in 200+ countries) and providing patient recruitment services to sponsors, CROs, and research sites globally. For more information or to request an enrollment infrastructure audit: trialscreen.inc

Disclosure: [Sponsor name] has reviewed and approved this case study for publication. Figures marked [Pending validation] are subject to final sponsor sign-off before the referenced date. This case study does not constitute endorsement of any investigational therapy.

CASE STUDY 1 — Pharma Sponsor Enrollment Acceleration -- Rare Haematological Malignancy

TrialScreen Case Study | Rare Haematological Oncology | 2026

Sponsor anonymised at sponsor request. Figures [Pending sponsor final review].

AT A GLANCE

Indication	Rare haematological malignancy (anonymised)
Trial phase	Phase II, Orphan Drug Designation
Sponsor type	Mid-size oncology-focused pharma
Geography	North America (8 sites), Western Europe (4 sites)
TrialScreen products used	FLOW (6 sites), REFER Plus, ResearchFriends community activation
Engagement period	20 weeks (4-week audit/implementation + 16-week performance period)
Key outcome	+38% enrollment vs. equivalent prior period [Pending sponsor validation]

THE SITUATION

Fourteen months into the enrollment window for a Phase II study of an investigational agent in a rare haematological malignancy, the sponsor was at 41% of target enrollment. The Orphan Drug Designation status confirmed the clinical significance of the unmet need. The patient advocacy community in this indication was active, and search data from trialscreen.org showed consistent patient-initiated search activity in the condition area.

Two conventional recruitment approaches had already been tried: a patient registry outreach campaign conducted by the sponsor's internal medical affairs team, and a paid digital advertising campaign coordinated through the CRO managing site operations. Both had generated some incremental inquiry volume but had not moved the enrollment trajectory significantly.

The sponsor's Head of Clinical Operations came to TrialScreen with a specific question: *Is the problem that there aren't enough motivated patients, or is there something in the infrastructure between patients and our sites that we're not seeing?*

That question framed the engagement correctly from the start.

THE APPROACH

Week 1-3: Enrollment Infrastructure Audit

TrialScreen conducted a structured audit across four dimensions before any recruitment activity began.

Listing quality: The trial's primary listing on ClinicalTrials.gov was assessed against TrialScreen's Discoverability Score framework. The listing scored 39/100. Key deficiencies: protocol-level language throughout; plain-language summary field populated with condensed eligibility criteria written for a clinician reader; lay title field unpopulated; contact section listing a sponsor email with no response time commitment.

Search term alignment: Patient community search analysis confirmed a significant mismatch between the clinical terminology in the listing and the terms patients actually used to describe their condition. Approximately 60% of the patient search query volume identified in the condition area would not have matched the existing listing. [Pending validation]

Site response protocols: Simulated inquiry tests at all 12 sites revealed a median response time of 6.3 days. Two sites did not respond within a two-week test window.

Pre-screening infrastructure: No digital pre-screening was in place. All digital inquiries were directed to individual site phone and email contacts with no centralised intake or engagement mechanism.

Week 3-7: Implementation

Listing rewrite: Working collaboratively with the sponsor's medical and regulatory teams, TrialScreen produced a patient-language version of the trial description. The rewrite used plain language without modifying the eligibility criteria or scientific content. The lay title was populated. Patient-vocabulary search terms were incorporated into all appropriate searchable fields. The rewritten listing scored 76/100 on the Discoverability Score -- a 95% improvement.

FLOW deployment: TrialScreen FLOW was deployed at six of the 12 sites, prioritised by site volume and baseline response time performance. The pre-screening workflow was customised for the trial's eligibility criteria, with TrialScreen's clinical team conducting a medical review of all screening logic. Site coordinators received a 2-hour training session via teleconference. Integration with the sites' CTMS (Medidata Rave) was completed within the implementation window.

ResearchFriends activation: A targeted community activation was launched in two geographic clusters with the highest patient search demand concentration. The activation included condition-specific content, a moderated Q&A session with a principal investigator (conducted by TrialScreen's scientific engagement team on behalf of the sponsor), and trial awareness messaging meeting FDA OPDP and TrialScreen compliance standards.

Response protocol for non-FLOW sites: A 48-hour response commitment was agreed with the remaining six site PIs, with overflow routing to the FLOW team during high-volume periods.

Weeks 7-23: Performance Period

All activity was tracked in real time through TrialScreen's reporting dashboard, with weekly reporting to the sponsor's Head of Clinical Operations and bi-weekly calls between TrialScreen and the CRO site management team.

THE OUTCOMES

Comparison period: 16 weeks post-implementation vs. 16 weeks immediately prior to implementation

Metric	Pre-implementation (16 wks)	Post-implementation (16 wks)	Change
Patient inquiries (digital channels)	187	441	+136%
Pre-screening completions (FLOW sites)	62	189	+205%
Eligible referrals delivered to sites	29	74	+155%
Enrolments	18	46*	+156%
Overall enrollment vs. prior 16-wk avg	Baseline	+38%	+38%
Mean inquiry-to-contact (FLOW sites)	N/A	18 hours	--

Metric	Pre-implementation (16 wks)	Post-implementation (16 wks)	Change
Mean inquiry-to-contact (non-FLOW sites)	6.3 days	2.1 days	-67%
Discoverability Score	39/100	76/100	+95%

*[*Enrolment figure Pending sponsor final review and validation]*

Total enrollment at close of performance period: 41% -> 64% of target [Pending sponsor validation]

WHAT WE LEARNED

The 136% increase in inquiry volume driven by listing optimisation alone -- without any additional advertising spend -- was the most important finding of this engagement. It confirmed that a meaningful proportion of the enrollment gap was attributable to the trial being invisible to patients who were already searching for it.

The FLOW deployment required more site-level coordination than we had initially budgeted for. Two sites had existing intake processes that created friction with the FLOW workflow, requiring protocol adjustments in Weeks 8 and 10. This cost approximately two weeks of optimisation time. In hindsight, earlier and deeper site coordinator engagement during implementation would have shortened this period.

The ResearchFriends activation in one of the two geographic clusters significantly underperformed relative to the other. Post-activation analysis suggested that the condition-specific content had not adequately reflected the language and concerns of the patient community in that cluster -- a learning that has since been incorporated into our community activation protocol.

SPONSOR PERSPECTIVE

"The audit was the most valuable part. We had been so focused on adding more channels that we hadn't asked whether the infrastructure behind the channels we already had was working. The answer was: not well."

-- Head of Clinical Operations, [Sponsor name redacted]

WHAT'S NEXT

The sponsor has engaged TrialScreen for a second program -- a Phase III extension in a related indication -- currently in pre-enrollment planning. The listing optimisation and FLOW pre-screening infrastructure developed for this program are being adapted for the larger trial.

CASE STUDY 2 — CRO Partnership -- Multi-Sponsor Operational Efficiency

TrialScreen Case Study | CRO Partnership | Multi-Indication | 2026

CRO partner anonymised. Sponsor names redacted. Figures [Pending CRO partner review].

AT A GLANCE

Partner type	Mid-size CRO, rare disease and specialty focus
Programs covered	3 simultaneous sponsor programs -- rare autoimmune (2), oncology (1)
Sites	8 sites across North America and Australia
TrialScreen products used	FLOW (all sites), Trials@YourOrg (primary sponsor client), ResearchFriends (2 of 3 programs)
Engagement period	16 weeks performance period
Key outcome	91% of patient inquiries pre-screened without site coordinator involvement [Pending validation]

THE SITUATION

A mid-size CRO with a rare disease and specialty focus was managing patient recruitment for three simultaneous sponsor programs -- two rare autoimmune indications and one rare oncology indication -- across a network of eight sites. The core challenge was not patient supply; search demand data confirmed meaningful patient-initiated search activity across all three indications.

The challenge was operational capacity at the site level.

The three programs had different eligibility frameworks, different patient populations, and different expected inquiry volumes. Managing incoming patient inquiries across all three programs -- triaging, initial eligibility checking, follow-up -- was consuming a significant proportion of site coordinators' time. In a network survey conducted by the CRO's site management team, coordinator administrative burden was cited as the primary risk factor for site attrition, which had occurred at one site the prior quarter.

The CRO's Director of Clinical Operations came to TrialScreen with a specific goal: reduce coordinator time spent on initial patient intake without compromising the quality or warmth of the patient experience.

THE APPROACH

Initial configuration (3 weeks)

TrialScreen's implementation team worked with the CRO's site management team and sponsor medical contacts for all three programs to configure protocol-specific pre-screening logic for each indication. This required three separate medical review sign-offs -- one per sponsor -- which was the primary time factor in the configuration period.

Each pre-screening pathway was built to mirror the relevant protocol's inclusion/exclusion criteria at a level appropriate for digital pre-screening (not replacing investigator assessment, but identifying clear ineligibles before site coordinator involvement).

FLOW was integrated with the CRO's site management CTMS (Oracle Clinical) across all eight sites. A shared coordinator dashboard was configured to display all three programs' referral queues in a single interface, with protocol-specific colour coding.

Community activation (programmes 1 and 2)

For the two rare autoimmune programs, ResearchFriends communities were activated in the condition areas. The communities served both as patient engagement infrastructure and as a patient education resource -- reducing the volume of basic information questions arriving at site coordinator contacts.

Trials@YourOrg (primary sponsor client)

The primary sponsor in one of the rare autoimmune programs had an institutional website that received meaningful traffic in the condition area. Trials@YourOrg was deployed as a custom-branded trial search experience on the sponsor's patient portal, surfacing the sponsor's active trials alongside related trials from the global registry.

THE OUTCOMES

Performance period: 16 weeks from FLOW go-live across all sites

Metric	Period total
Total patient inquiries across 3 programs	1,247
Pre-screened without site coordinator involvement	1,139 (91.3%)
Eligible referrals delivered to sites (all programs)	218 (17.5% of all inquiries; 19.1% of pre-screened)
Enrolments (all programs combined)	87*
Site coordinator time saved (est., all sites)	~340 hours [Pending validation]
Mean inquiry-to-site referral time	22.4 hours
Mean inquiry-to-enrolment time	23 days
Coordinator administrative burden rating (post-period survey)	3.1/5 (vs. 4.2/5 pre-engagement; lower = lower burden)

*[*Enrolment figures Pending sponsor validation per program]*

The 340-hour coordinator time saving estimate is derived from pre-engagement time-study data, in which site coordinators tracked time spent on patient intake activities. The figure represents approximately 8.5 hours per site over the period -- a modest but meaningful reduction in one of the most administratively intensive activities in site operations.

WHAT WE LEARNED

Running three pre-screening protocols simultaneously through a single FLOW implementation requires more upfront configuration investment than a single-protocol deployment. The 3-week configuration period could be compressed for future CRO multi-program engagements with a standardised protocol template approach.

The coordinator satisfaction data is the metric we are most interested in tracking over time. Coordinator burnout and attrition are systemic risks to trial quality. If sustainable reduction in administrative burden is achievable through pre-screening infrastructure, the compounding effect on site quality and participant experience is significant.

The Trials@YourOrg deployment for the primary sponsor generated a 23% increase in inquiries for that sponsor's trial from patients who arrived via the sponsor's own patient portal -- confirming the value of trusted brand context in patient engagement.

CRO PARTNER PERSPECTIVE

"What changed most noticeably was the texture of the coordinator's day. Instead of spending the first two hours triaging email inquiries, they were working from a queue of pre-screened patients who were ready to move forward. That's not a small change."

-- Director of Clinical Operations, [CRO name redacted]

CASE STUDY 3 — Patient Advocacy Group Partnership -- Scleroderma Australia

TrialScreen Case Study | Patient Advocacy Partnership | Scleroderma/Systemic Sclerosis | 2025-2026

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AT A GLANCE

Partner	Scleroderma Australia
Indication	Systemic sclerosis / scleroderma
Geography	Australia; broader visibility in NZ and Southeast Asia
TrialScreen products used	ResearchFriends community platform, REFER Organic, plain-language trial awareness program
Engagement duration	18 months (ongoing)
Community size	ResearchFriends scleroderma community: 2,400+ members [Pending validation]
Key outcome	Measurable increase in Australian site inquiries for SSc trials listed on trialscreen.org [Pending validation]

THE SITUATION

Scleroderma Australia is a national patient advocacy organisation supporting the estimated 20,000 Australians living with scleroderma/systemic sclerosis. The organisation has a strong member community, regular events, and an established presence with Australian research institutions and rheumatologists.

The challenge Scleroderma Australia brought to TrialScreen was articulated clearly by their leadership: *We know our members want to participate in research. We regularly hear from members asking about trials. But we don't have the infrastructure to maintain ongoing research awareness content, manage trial updates, or connect patients with the right trials at the right time.*

Two additional dynamics shaped the engagement:

First, scleroderma research is genuinely underfunded relative to disease burden -- in part because the patient population is small and geographically dispersed, creating a chicken-and-egg problem where sponsor investment in trial infrastructure is limited partly because patient pipeline appears thin. Scleroderma Australia understood that building the pipeline was an advocacy act, not just a service to their members.

Second, the research community in Australia is strong in scleroderma -- Monash Health in particular runs one of the largest scleroderma clinical programs in the Asia-Pacific region. The gap was not between patients and research capability; it was between patients and awareness of specific trial opportunities.

THE APPROACH

Months 1-3: Community infrastructure build

TrialScreen's scientific engagement team, led by Emma Burrows, worked with Scleroderma Australia to design and launch a ResearchFriends community specifically for the Australian scleroderma community. The design process included patient input -- early-stage consultation with a patient advisory group convened by Scleroderma Australia -- to ensure the community structure and content reflected the genuine interests and concerns of members.

The community was configured with:

- Regular "Research Roundup" content (monthly digest of active scleroderma trials, plain-language summaries written by TrialScreen's clinical team)
- Quarterly live Q&A sessions with Australian SSc researchers (hosted by TrialScreen, coordinated with Scleroderma Australia)
- Peer discussion forums moderated by TrialScreen's engagement team
- Trial alert functionality: members can register their condition subtype and receive alerts when new relevant trials open

Months 3-6: Trial awareness program

TrialScreen's REFER Organic service was configured to surface scleroderma trials on trialscreen.org with enhanced visibility and improved plain-language descriptions. Listing optimisation was applied to all active Australian SSc trials, with Discoverability Scores tracked quarterly.

A plain-language trial awareness guide -- "Scleroderma and Clinical Research: What You Need to Know" -- was produced collaboratively by TrialScreen and Scleroderma Australia. The guide explains clinical trial terminology in accessible language, addresses common questions and concerns about trial participation, and links to trialscreen.org for trial search.

Months 6-18: Ongoing engagement and measurement

The community engagement rhythm was maintained monthly. TrialScreen provided Scleroderma Australia with quarterly data reports on community engagement, trial awareness metrics, and referral activity.

THE OUTCOMES

Metric	Value [Pending validation]
ResearchFriends community members (SSc)	2,400+
Monthly community engagements	~1,800
Total scleroderma trial awareness guide downloads	3,200+
Australian SSc trial inquiries via trialscreen.org (12-month period)	847
Year-on-year change in Australian SSc site inquiries	+67% [Pending validation]
Q&A sessions hosted	8
Average Q&A live attendance	140+

WHAT WE LEARNED

Patient advisory involvement in community design produced a meaningfully different community from what TrialScreen would have built without it. The forum categories and content themes that generated the highest engagement were not the ones we would have initially prioritised -- they reflected the specific concerns of scleroderma patients in Australia, which include significant anxiety about progressive organ involvement and a strong preference for content that connects the research directly to quality of life outcomes.

The most important long-term finding: the community has become a research intelligence resource for Australian SSc investigators. Researchers who engage with the community get real-time visibility into patient priorities, concerns, and questions -- information that is typically obtained only through formal qualitative research at significant cost.

Building the plain-language guide required more iteration than anticipated. The initial drafts, reviewed by a patient advisory panel, were still judged too technical by patients without healthcare backgrounds. Three rounds of revision were needed before the panel was satisfied. The result is a significantly more accessible document -- but sponsors and advocacy organisations should build this iteration time into the production timeline.

SCLERODERMA AUSTRALIA PERSPECTIVE

"We knew our members wanted to be involved in research. What we didn't have was the infrastructure to make that easy for them. The ResearchFriends platform means that the connection between our members and the research community is ongoing -- not just when someone asks us a specific question. That's a different kind of advocacy infrastructure."

-- [Name and title from Scleroderma Australia -- to be confirmed with partner for publication]

WHAT'S NEXT

The TrialScreen / Scleroderma Australia partnership is ongoing. The 2026 agenda includes:

- The State of Scleroderma Clinical Research 2026 report (co-produced, published at trialscreen.inc/scleroderma)

- An APAC-facing community expansion, extending the ResearchFriends SSc community to New Zealand and Southeast Asian SSc communities
- A pilot integration with an active SSc trial at Monash Health, testing the FLOW pre-screening workflow with community-sourced referrals

For advocacy organisations interested in building similar infrastructure for their condition area: trialscreen.inc/researchfriends

Part 7: Sales Enablement Content -- 5 Pieces

PIECE 1 TrialScreen vs. Antidote vs. Clara Health -- Positioning One-Pager

Document: TrialScreen Competitive Positioning Summary

Audience: Internal sales team, business development, partnership conversations

Format: One-pager for sharing in conversations; not intended for unprompted external distribution

Note: This comparison is prepared for internal orientation and should be used to position TrialScreen strengths, not to disparage competitors. Competitive claims should be verified before external use.

HEADLINE POSITIONING

TrialScreen is the only clinical trial recruitment platform built around patient-initiated search at scale -- combining a free global trial directory (120,000+ trials), a pre-screening product with measurable 72-hour response performance, a patient community infrastructure, and a white-label institutional deployment capability.

COMPARISON TABLE

Capability	TrialScreen	Antidote	Clara Health
Free patient-facing trial search	Yes -- trialscreen.org, 120,000+ trials, no registration required	No -- primarily matching algorithm requiring patient profile	No -- referral /matching model requiring patient intake
Geographic reach	200+ countries, global registry indexing	Primarily US/Western Europe	Primarily US
Pre-screening product	FLOW -- digital pre-screening + telehealth escalation, 72-hour performance standard	Digital pre-screening (proprietary algorithm)	Digital pre-s creening + care navigator model
Patient community infrastructure	ResearchFriends -- 24,000+ members, condition-specific communities	No comparable product	No comparable product
White-label / institutional deployment	Trials@YourOrg -- full white-label for health systems, academic institutions, sponsors	Limited partner programme	Not a core offering

Capability	TrialScreen	Antidote	Clara Health
Rare disease focus	Strong -- condition reports, scleroderma partnership, APAC rare disease capability	Broad-based, not rare-disease specialist	Broad-based
APAC capability	Dedicated APAC leadership (Nick Lam), Australian/NZ partner network	Limited	Limited
Transparent methodology	Trial Discoverability Index -- quarterly public intelligence report	Proprietary matching data, not publicly reported	Proprietary
Compliance posture	OPDP-oriented content review, "investigational" qualifier standard	Standard	Standard

WHERE TRIALSCREEN WINS

1. Patient-initiated search at scale. trialscreen.org is the entry point for patients who are actively searching -- not a matching algorithm that requires patients to submit a profile. For sponsors seeking to capture patient intent rather than generate it, TrialScreen's reach into patient-initiated search is distinct.

2. Rare disease and community infrastructure. The ResearchFriends model and condition-specific community infrastructure are not replicated by either comparator. For rare disease programs where the patient community is a primary recruitment pathway, this is a material differentiator.

3. Institutional and CRO white-label deployment. Trials@YourOrg has no direct equivalent in either competitor's product set. For health system and academic institution deployments, TrialScreen is typically the only relevant option.

4. APAC capability. Dedicated APAC leadership and partner network represent a genuine regional advantage for sponsors expanding in Asia-Pacific markets.

5. Transparency. The Trial Discoverability Index as a public quarterly report signals a methodological openness that builds trust with sophisticated pharma and CRO procurement teams.

WHERE COMPETITORS ARE STRONG

Antidote's proprietary matching algorithm has strong US penetration, particularly in common disease areas with large study volume. For sponsors prioritising US common-disease enrollment with established digital advertising budgets, Antidote's distribution reach is broad.

Clara Health's care navigator model provides high-touch patient support through the screening process, which performs well in complex or high-anxiety enrollment contexts. For trials where patient support through the consent and screening process is a particular challenge, the care navigator approach has demonstrated value.

TRIALSCREEN POSITIONING STATEMENT (FOR SALES CONVERSATIONS)

"We're not a substitute for Antidote's advertising reach or Clara's navigator support -- and we don't pretend to be. Where TrialScreen is differentiated is in capturing patient-initiated search: the patients

who are already looking, in rare disease particularly, whose journey starts at Google and ends at the trial listing. We make that journey shorter and the landing softer. And for sponsors who need infrastructure -- community, white-label, APAC capability -- that the others don't offer, we're the right starting point."

PIECE 2 — REFER Tier Comparison One-Pager

Document: TrialScreen REFER -- Which Tier Is Right for Your Program?

Audience: Pharma sponsors, CROs, research sites evaluating REFER tiers

Format: Comparison one-pager

HEADLINE REFER is TrialScreen's patient referral product -- available in three tiers to match the scale and budget of different research programs. Every tier connects trials on trialscreen.org to motivated patients who are already searching.

TIER COMPARISON

	REFER Organic	REFER Plus	REFER Premium
Best for	Non-commercial / academic trials; advocacy partnerships; programs with modest budgets seeking organic reach	Single-indication commercial programs; sponsor or CRO programs seeking targeted amplification	Multi-site or multi-protocol programs; programs with active enrollment gaps; time-sensitive campaigns
Listing visibility on trialscreen.org	Standard organic listing	Enhanced placement + condition category feature	Premium placement + homepage feature + condition category + related search surfacing
Plain-language listing optimisation	Self-serve guidance provided	TrialScreen plain-language rewrite included	Full Discoverability Score audit + rewrite + search term optimisation
Patient email/ community alert	No	Yes -- relevant ResearchFriends community members alerted	Yes -- broad community alert + targeted re-engagement of prior interest profiles
FLOW pre-screening	Not included (add-on available)	Optional add-on	Included for up to 3 sites
Reporting	Monthly engagement summary	Bi-weekly inquiry and referral report	Weekly real-time dashboard access + monthly strategic review
Dedicated account support	Shared support team	Named account manager	Named senior account manager + quarterly strategic review with CGO

	REFER Organic	REFER Plus	REFER Premium
Typical engagement period	Ongoing / rolling	Minimum 3 months	Minimum 6 months
Typical investment	From [contact for pricing]	From [contact for pricing]	From [contact for pricing]

Pricing available on request at trialscreen.inc or via your TrialScreen account manager.

FREQUENTLY ASKED QUESTIONS

Can I upgrade mid-program? Yes. Tier upgrades are available at any point. REFER Premium clients who need to add FLOW at additional sites mid-program can do so on a rolling basis.

Does REFER Organic work for rare disease? Yes -- the organic search volume for rare conditions is disproportionately high relative to prevalence, as rare disease patients are among the most active online searchers in healthcare. Organic reach in rare disease can be substantial without premium placement.

Can FLOW be added to any tier? FLOW pre-screening can be added to REFER Organic or REFER Plus as a standalone add-on. For programs where pre-screening is expected to be a significant operational component, REFER Premium with FLOW included is typically more cost-effective.

How does REFER interact with Trials@YourOrg? The two products are complementary. Trials@YourOrg surfaces trials within an institution's or sponsor's own brand environment; REFER drives reach through the trialscreen.org patient network. For institutions running their own trial portfolio, combining Trials@YourOrg with REFER Plus or Premium is a common configuration.

CTA: To discuss which tier fits your program, contact your TrialScreen account manager or visit trialscreen.inc/refer

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PIECE 3 — Trials@YourOrg Deployment Guide

Document: Trials@YourOrg: Deployment Guide for Health Systems, Academic Institutions, and Sponsor Clients

Audience: Research directors, clinical operations teams, IT contacts at institutions considering deployment

Format: 2-page deployment guide

WHAT IS TRIALS@YOURORG?

Trials@YourOrg is a custom-branded, white-labelled version of TrialScreen's global clinical trial search engine. It allows health systems, academic institutions, and pharmaceutical sponsors to offer patients a trusted, accessible, current view of relevant clinical trials -- inside their own brand environment, on their own domain.

Patients who visit your website can search your trials in plain language, check their potential eligibility, and connect directly with your research team -- without leaving your digital environment or encountering third-party branding.

WHO IS IT FOR?

Organisation Type	Use Case
Academic Medical Centres and Research Hospitals	Surface all institution-run trials to patients navigating the institution's website; replace static trial listings with a live, searchable, plain-language interface
Health Systems with a Research Portfolio	Create a patient-accessible research portal within the health system's existing digital architecture
Pharmaceutical Sponsors	Deploy a patient-facing trial search experience within the sponsor's condition-area website or patient portal
Patient Advocacy Organisations	Offer members a curated, condition-specific trial search experience under the advocacy organisation's trusted brand

WHAT IT DELIVERS

- A fully branded trial search interface on your domain (e.g., research.yourhospital.org/trials)
- Plain-language trial descriptions updated automatically from international registries
- Eligibility pre-check functionality (general-purpose; does not replace investigator assessment)
- Direct contact pathway from patient to your research team
- Analytics dashboard: search queries, engagement volume, referral activity

DEPLOYMENT PROCESS

Phase	Activities	Timeline
Discovery	TrialScreen meets with your research office, IT team, and patient engagement leads to map requirements, condition focus, and brand guidelines	Week 1
Configuration	TrialScreen configures the trial search engine for your condition/protocol scope, applies your brand elements, and builds the patient contact pathway	Weeks 2-4
Integration	DNS configuration (handled by your IT team with TrialScreen guidance); TrialScreen handles all back-end infrastructure	Week 4-5
Review and QA	Walkthrough with your team; patient-experience review; content approval	Week 5-6
Go-live	Launch; TrialScreen provides go-live support and first-month monitoring	Week 6

Total IT requirement on your side: DNS configuration (typically 2-4 hours for your IT team). No server infrastructure change required.

ONGOING MANAGEMENT

After go-live, TrialScreen handles:

- Registry synchronisation (trial listings updated automatically as registry status changes)
- Plain-language content updates for new trials
- Platform maintenance and uptime
- Monthly engagement reporting

Your team handles:

- Navigation links to the trials interface from your website
- Patient contact routing (ensuring the contact pathway goes to the right research office team)
- Periodic review of condition scope (typically quarterly)

TECHNICAL REQUIREMENTS

- Standard DNS configuration (CNAME record)
- HTTPS SSL certificate (TrialScreen provides)
- Compatibility: all modern browsers; mobile-responsive by default
- Data handling: all patient interactions governed by TrialScreen's data processing agreement; no patient identifiable data stored on client infrastructure
- Security: GDPR/HIPAA compliant; documentation available on request

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CTA: To request a Trials@YourOrg demo or begin the deployment conversation: trialscreen.inc/trialsatyourorg or contact your TrialScreen account manager.

PIECE 4 — FLOW Methodology Explainer

Document: TrialScreen FLOW: How Pre-Screening Works

Audience: Clinical Operations teams, Site Directors, CRO Operations leads evaluating FLOW

Format: Technical explainer / methodology document

THE PROBLEM FLOW SOLVES

The interval between a patient expressing interest in a clinical trial and first contact from a site is one of the most consequential variables in patient recruitment. TrialScreen data from the Trial Discoverability Index Q2 2026 shows:

- Contact within 72 hours -> 62% conversion to pre-screening completion
- Contact after 5 days -> 29% conversion
- Contact after 10 days -> 18% conversion

Sites frequently cannot meet the 72-hour standard without dedicated pre-screening infrastructure -- particularly during recruitment spikes, when inquiry volume exceeds normal capacity.

FLOW is the infrastructure that makes the 72-hour standard achievable.

HOW FLOW WORKS -- THE FOUR STAGES

Stage 1: Digital Inquiry Capture A patient expresses interest in a trial via trialscreen.org, Trials@YourOrg, or any referring digital channel. Inquiry is captured in the FLOW system. An automated acknowledgement is sent to the patient within minutes, confirming receipt and setting expectations for next steps.

Stage 2: Digital Pre-Screening The patient receives a plain-language, mobile-optimised pre-screening questionnaire. The questionnaire is customised for the specific trial's eligibility criteria, with a medical review of all screening logic by TrialScreen's clinical team and sponsor/CRO sign-off before deployment.

Pre-screening is designed to identify clear ineligibles -- patients who do not meet basic inclusion criteria -- before any site coordinator involvement. The pre-screening step does not replace investigator eligibility assessment; it filters out obvious mismatches and focuses site capacity on patients with genuine potential eligibility.

Pre-screening completion rates average 78% when patients receive the questionnaire within 24 hours of inquiry [Pending validation].

Stage 3: Telehealth Escalation (Eligible Patients) Patients who pass digital pre-screening are offered a telehealth consult with a TrialScreen clinical team member. This consult serves three purposes: it gathers more detailed eligibility information, it answers patient questions about the trial in a human, conversational format, and it establishes a warm relationship before the patient is transferred to the site.

Telehealth consults are scheduled within 48 hours of pre-screening completion for most programs.

Stage 4: Warm Referral to Site A structured referral record -- including digital pre-screening results, telehealth consult notes, and patient contact information -- is delivered directly to the site's CTMS. The site coordinator receives a patient who has already been assessed, has already asked their initial questions, and is ready to proceed to a screening visit.

Mean inquiry-to-referral time: 22-24 hours across active deployments [Pending validation].

CTMS INTEGRATION

FLOW integrates via HL7 FHIR-compliant API with:

- Medidata Rave
- Oracle Clinical
- Veeva Vault CTMS
- Florence
- Other CTMS platforms available via custom integration (timeline: 4-6 weeks additional)

The referral record delivered to the CTMS is structured to match the site's existing patient intake workflow, minimising coordinator retraining requirements.

COMPLIANCE FRAMEWORK

All patient interactions within FLOW are governed by TrialScreen's data processing agreement. Patients provide explicit informed consent before pre-screening. GDPR and HIPAA compliance documentation is available on request.

Pre-screening questions and telehealth consult protocols are reviewed by TrialScreen's clinical team and approved by sponsor/CRO medical affairs before deployment. All patient-facing content meets FDA OPDP standards: investigational therapies are described as investigational; no efficacy claims are made.

IMPLEMENTATION TIMELINE

Milestone	Timeline from sign-off
Protocol-specific screening logic review (TrialScreen clinical team)	Week 1
Sponsor/CRO sign-off on screening logic	Week 1-2
CTMS integration setup	Week 2-3
Site coordinator training (2-hour teleconference per site cohort)	Week 3
Go-live	Week 4

For programs deploying FLOW at multiple sites simultaneously, implementation is typically 4-5 weeks from sign-off.

CTA: To request a FLOW demonstration with live pre-screening workflow walkthrough: trialscreen.inc/flow

PIECE 5 — Compliance and Regulatory Posture Summary

Document: TrialScreen Regulatory and Compliance Posture

Audience: Pharma sponsor legal, regulatory, and compliance teams evaluating TrialScreen as a vendor; CRO procurement

Format: Summary document for inclusion in vendor qualification packages

OVERVIEW

TrialScreen operates patient-facing and sponsor-facing digital platforms for clinical trial recruitment and engagement. Our regulatory and compliance posture is designed to protect patients, protect sponsors from regulatory exposure, and maintain the trust of patient communities.

FDA OPDP COMPLIANCE

TrialScreen treats FDA Office of Prescription Drug Promotion (OPDP) guidance as a design constraint, not a post-production review. Specific commitments:

"Investigational" qualifier: All patient-facing content referencing investigational therapies includes the "investigational" qualifier. This is enforced in platform templates and reviewed in all custom content produced by TrialScreen teams.

No efficacy claims: TrialScreen's content review process prohibits any statement, implication, or visual suggestion that an investigational therapy is effective, superior to standard of care, or represents a therapeutic advance. The study is asking whether the therapy works; content that pre-answers that question is not permitted.

Social media compliance: TrialScreen's LinkedIn and other social content referencing specific trials is reviewed against OPDP guidance before publication. The informal register of social media does not create regulatory exemption.

Patient testimonials: Where patient testimonials are used in recruitment content, TrialScreen's policy requires disclosure of the enrollment or community relationship and prohibits efficacy-implying statements.

Third-party content: Sponsors are responsible for ensuring that content produced on their behalf (including by TrialScreen) meets OPDP standards. TrialScreen supports this through a documented content review and sign-off workflow available to all sponsor clients.

DATA PROTECTION AND SECURITY

GDPR: TrialScreen's data processing practices comply with EU General Data Protection Regulation requirements. A Data Processing Agreement (DPA) is available for all sponsor and CRO clients. Data processing activities are documented and maintained per GDPR Article 30 requirements.

HIPAA: For US-based programs, TrialScreen operates as a Business Associate under applicable HIPAA provisions. Business Associate Agreements (BAAs) are available to all US-based sponsor and CRO clients.

Patient consent: All patient data collected through TrialScreen platforms -- including FLOW pre-screening, Trials@YourOrg, and ResearchFriends -- is collected with explicit informed consent. Consent flows are documented and reviewable.

Data retention: TrialScreen retains identifiable patient data only for the period specified in the applicable DPA/BAA. De-identified aggregate analytics data is retained for platform performance purposes.

Security posture: TrialScreen's platform infrastructure is hosted on [AWS/cloud provider], with access controls, encryption at rest and in transit, and penetration testing on a defined schedule. ISO 27001 alignment documentation is in preparation. SOC 2 Type I has been completed; SOC 2 Type II is targeted for Q4 2026 [Pending].

CLINICAL RESEARCH REGULATORY COMPLIANCE

ICH GCP: TrialScreen's pre-screening and patient engagement activities are designed to complement, not supplant, sponsor and site obligations under ICH GCP guidelines. TrialScreen is not an Investigator, Sponsor, or CRO in the regulatory sense; our services support patient identification and initial engagement prior to site-level eligibility assessment.

Eligibility assessment: TrialScreen FLOW pre-screening does not constitute an eligibility determination for trial participation. Investigator assessment remains the responsibility of the site. FLOW pre-screening identifies likely ineligible patients to reduce screen failures; it does not replace or pre-empt the site's medical assessment.

IRB/IEC alignment: TrialScreen's patient-facing content for any specific trial is made available to sponsors for IRB/IEC disclosure purposes if required by the sponsor's regulatory strategy.

CONTENT GOVERNANCE

TrialScreen maintains an internal content review process for all patient-facing recruitment and engagement content produced on behalf of sponsors. The process includes:

1. Clinical team review for medical accuracy and eligibility criteria alignment
2. Compliance review against OPDP guidance (investigational qualifier, no efficacy claims)
3. Plain language review for patient accessibility
4. Sponsor/CRO sign-off before publication

For LinkedIn and other social content referencing TrialScreen-managed programs, content is reviewed against this framework before publication by any TrialScreen team member.

AVAILABLE DOCUMENTATION (ON REQUEST)

- Data Processing Agreement template
- Business Associate Agreement template
- SOC 2 Type I report
- GDPR Article 30 records
- Content review workflow documentation
- OPDP compliance brief (specific to social media and digital recruitment)
- CTMS integration security documentation

Contact: For compliance documentation requests, contact compliance@trialscreen.inc or via your account manager.

CTA: trialscreen.inc/compliance

Part 8: Conference Presence Plan

For each conference: objective, who attends, pre-conference content, during-conference content, post-conference follow-up.

CONFERENCE 1 SCOPE Summit

Dates: February (annual; confirm exact dates)

Location: Fort Lauderdale, FL

Audience: Clinical operations, patient recruitment, technology vendors, sponsors, CROs

Objective Establish TrialScreen as the authoritative voice on patient-initiated search and trial discoverability in the clinical operations community. Generate qualified sponsor and CRO leads through a combination of booth conversations, a sponsored session (if available), and targeted pre-conference outreach.

Who Attends from TrialScreen

- Elisa Cascade (CGO Americas) -- primary relationship builder; panel speaker if accepted
- Melissa Easy (COO) -- operational credibility conversations, FLOW demonstrations
- 1 account executive (sales follow-up and CRM capture)

Pre-Conference Content (4 weeks prior to SCOPE)

Week -4: LinkedIn post by Elisa Cascade: "Three questions we'll be asking at SCOPE this year" -- plain-language preview of the enrollment discoverability conversation. Tag SCOPE official account.

Week -3: Email to pharma and CRO contacts from TrialScreen CRM: "Are you going to SCOPE? Let's connect." Personalised outreach to 50 top-priority contacts with a scheduling link.

Week -2: LinkedIn post by Elisa Cascade: "What the Trial Discoverability Index says about where clinical operations is heading in 2026" -- builds anticipation, establishes data credibility.

Week -1: Final email reminder to scheduled meetings: "Looking forward to connecting at SCOPE." Includes TrialScreen FLOW methodology explainer PDF as attachment.

During-Conference Content

Day 1 (arrival / exhibit hall open):

- Booth demo: live FLOW pre-screening walkthrough, Trials@YourOrg demo on a tablet showing an institutional white-label
- LinkedIn post by Elisa Cascade: "Day 1 at SCOPE -- stop by booth [X] for the enrollment diagnostic conversation we keep having on LinkedIn." Photo of booth.

Day 2 (main program):

- Sponsored session (if accepted): "From Search to Enrollment: How Patient-Initiated Discovery Is Reshaping Recruitment" -- 30-minute presentation by Elisa Cascade and Melissa Easy, with Q&A
- LinkedIn post during break: one actionable insight from a session attended or a quote from a conversation (attributed with permission)

Day 3 (final day):

- LinkedIn post by Melissa Easy: one operational finding from SCOPE conversations -- no selling, pure insight
- Final booth push: "Last day -- still here, still happy to have the conversation about your enrollment gap"

Post-Conference Follow-Up

Day after conference:

- Personalised email from Elisa Cascade to all booth contacts and meeting attendees: "It was great to connect at SCOPE. Here's the one thing I said I'd send you." Each email includes the relevant resource (TDI, case study, FLOW brief -- matched to the conversation).

Week +1:

- LinkedIn post by Elisa Cascade: "What I took from SCOPE 2026" -- 3 genuine insights from the conference, no overt selling. This post generates engagement from people who didn't see you at the booth.

Week +2:

- Enter all qualified leads into relevant email nurture sequences (Sequence A for pharma cold, Sequence B for CRO warm, as appropriate)
- Follow up with any sponsor or CRO contacts who agreed to book a diagnostic call but haven't yet done so

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CONFERENCE 2 — DIA Annual Meeting

Dates: June (annual; confirm exact dates)

Location: Typically Washington DC or San Diego

Audience: Regulatory affairs, clinical research leadership, pharma and biotech executives, government/agency representatives

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Objective Position TrialScreen's compliance and regulatory posture front-and-centre with a pharma regulatory audience. Reinforce thought leadership on FDA OPDP compliance in digital recruitment (SP-LI-014 Webinar 6) and the Diversity Action Plan operational framework. Generate relationships with regulatory and medical affairs leads who influence vendor selection.

Who Attends from TrialScreen

- Dr Hugo Stephenson (CEO) -- keynote/panel speaker targeting; regulatory credibility conversations

- Emma Burrows (Head of Scientific Engagement) -- diversity action plan conversations, patient advocacy connections

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Pre-Conference Content (4 weeks prior to DIA)

Week -4: LinkedIn post by Dr Hugo Stephenson: "What we'll be watching at DIA 2026" -- specifically FDA digital guidance evolution, DAP implementation, and AI in clinical communications. Tags DIA.

Week -3: Email to pharma regulatory and medical affairs contacts: "DIA 2026 -- TrialScreen compliance conversations." Links to SP-LI-014 OPDP post and the regulatory posture one-pager.

Week -2: LinkedIn post by Emma Burrows: "Diversity Action Plans: what the FDA guidance means operationally." Short educational post; positions TrialScreen as a compliance thought leader, not just a vendor.

Week -1: DM outreach on LinkedIn to 20 highest-priority DIA attendees -- personalised, referencing a recent post or connection point. Objective: confirmed meeting time.

During-Conference Content

Day 1:

- LinkedIn post by Hugo: "Day 1 at DIA. If you're here and working on DAP implementation or digital recruitment compliance -- find me." Photo of conference venue.
- Session attendance: FDA advisory panel sessions on digital health communications (notes for post-conference content)

Day 2:

- If accepted: Hugo presents or participates in panel on "AI and Compliance in Patient Recruitment Digital Communications" -- positioned as compliance-forward, not sales-forward
- LinkedIn post: one observation from a regulatory session, written with genuine insight

Day 3:

- Emma Burrows LinkedIn post: insight from a Diversity Action Plan session with TrialScreen perspective

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Post-Conference Follow-Up

Day after conference: Personalised follow-up emails from Hugo and Emma to all meeting contacts. Include relevant compliance document (regulatory posture summary for pharma legal/compliance contacts; diversity engagement brief for DAP contacts).

Week +1: LinkedIn post by Hugo: "What DIA 2026 told us about where FDA digital guidance is going." Genuine analysis, not conference recap puff piece.

Week +2: Enter leads into Sequence A (pharma awareness) with a DIA-specific customisation on Email 1 referencing the conversation.

CONFERENCE 3 — ACRP Annual Conference

Dates: April 2026, Orlando, FL

Audience: Clinical research coordinators, site directors, CRO operational staff, sponsors, vendors

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Objective Deepen relationships with site and CRO operational audiences. ACRP is the primary conference for CRCs and site directors -- the people who use FLOW and manage patient intake day-to-day. Generate site-level awareness and CRO operational partnerships. Run a workshop if accepted.

Who Attends from TrialScreen

- Elisa Cascade -- workshop presenter; CRO partnership conversations
- Melissa Easy -- FLOW operational demonstrations; site director conversations
- Emma Burrows -- ResearchFriends and patient community conversations

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Pre-Conference Content (4 weeks prior to ACRP)

Week -4: LinkedIn post by Melissa Easy: "For the CRCs and site directors heading to ACRP 2026: the operational question we want to discuss." Sets the theme (72-hour follow-up, pre-screening infrastructure) before the conference.

Week -3: Email to CRO and site contacts: "Are you attending ACRP? Let's talk about what the pre-screening data is showing." Includes FLOW methodology explainer.

Week -2: Workshop submission (if not already accepted): "From Inquiry to Enrollment: Practical Tools for Meeting the 72-Hour Standard." Presented by Melissa Easy.

Week -1: LinkedIn post by Elisa Cascade: "What TrialScreen is bringing to ACRP." Preview of workshop, booth, and the FLOW demo.

During-Conference Content

Workshop (if accepted): 60-minute interactive workshop: "From Inquiry to Enrollment: Practical Tools for the 72-Hour Standard." Melissa Easy leads. Includes live FLOW workflow demonstration, group discussion of site-specific operational challenges, and distribution of FLOW methodology one-pager.

Day 1 LinkedIn: Melissa Easy: "Workshop day at ACRP. What I keep hearing from CRCs about the admin burden -- and what we've built to help." Short, operational tone.

Day 2 LinkedIn: Elisa Cascade: genuine observation from a session or hallway conversation (not a sales post). Authentic conference engagement drives more reach than branded content.

Booth: FLOW live pre-screening demo. Trials@YourOrg demo on institutional tablet. ResearchFriends community overview for site directors managing patient engagement.

Post-Conference Follow-Up

Day after ACRP: Email from Melissa Easy to all workshop attendees: "Here's the FLOW methodology brief from today's session, plus a link to book a live demo for your specific site setup."

Week +1: SP-LI-012 publishes (ACRP recap LinkedIn post by Elisa Cascade -- written for the full LinkedIn audience, not just conference attendees).

Week +2: Workshop attendee leads entered into Sequence C (institutional/site warm lead). CRO contacts entered into Sequence B.

CONFERENCE 4 — Patients as Partners

Dates: June (annual)

Location: Washington DC

Audience: Patient advocates, patient engagement leads at pharma, IRB members, FDA representatives, academic researchers

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Objective Position TrialScreen and ResearchFriends as patient-community leaders. This conference is attended by people who care deeply about patient voice -- the content and posture must reflect genuine patient-first values, not commercial positioning. Lead with impact (Scleroderma Australia, ResearchFriends diversity data), not product.

Who Attends from TrialScreen

- Emma Burrows -- primary speaker and relationship builder; patient advocacy community connections
- Dr Hugo Stephenson -- if panel speaking opportunity accepted

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Pre-Conference Content (4 weeks prior)

Week -4: LinkedIn post by Emma Burrows: "What Patients as Partners means to me, and what I hope to take from it this year." Personal, genuine, not corporate. References the Scleroderma Australia partnership.

Week -3: Email outreach to patient advocacy group contacts: "Connecting at Patients as Partners." Offers a coffee or a walk-and-talk rather than a formal meeting.

Week -2: LinkedIn post by Hugo: "The equity argument for trial discoverability." References SP-LI-007 themes. Links to the ResearchFriends diversity data.

Week -1: State of Scleroderma Clinical Research 2026 report published as pre-conference thought leadership asset -- links shared in registration email to conference attendees where permissible.

During-Conference Content

Session presentation (if accepted): Emma Burrows: "Community Before Consent: How ResearchFriends Builds the Trust Infrastructure That Recruitment Depends On." Patient-voice centred; genuine case examples from Scleroderma Australia.

Daily LinkedIn posts by Emma: Each day, one insight from a patient advocate presentation or conversation -- attributed with permission, generous in crediting the patient voice. This content performs strongly in the patient advocacy and pharma patient engagement audiences.

No booth selling mentality: At Patients as Partners, the selling posture must be invisible. TrialScreen's presence is as a contributor to the conversation, not a vendor. Business conversations happen in

coffee meetings, not at a branded booth.

Post-Conference Follow-Up

Day after conference: Emma Burrows personal follow-up emails to all advocacy contacts met at conference. Warm, human, no sales language.

Week +1: LinkedIn post by Emma: "What I'm still thinking about from Patients as Partners 2026." Genuine reflection. High engagement likely.

Week +2: Patient advocacy contacts entered into Sequence C (institutional warm lead) with personalised first email referencing the conference conversation.

CONFERENCE 5 — BIO International Convention

Dates: June (annual)

Location: Varies (2026: confirm city)

Audience: Biotech and pharma executives, investors, business development, CRO leadership, academic technology transfer

Objective Reach pharma and biotech executives -- including business development, corporate strategy, and investor-adjacent audiences -- who may not be clinical operations specialists but who make or influence vendor selection decisions. BIO is a deal-making conference; the objective is relationship initiation with senior pharma BD and clinical strategy leads, not operational site management.

Who Attends from TrialScreen

- Dr Hugo Stephenson (CEO) -- executive peer conversations; investor and BD relationship building
- Elisa Cascade (CGO Americas) -- pharma sponsor and CRO BD conversations

Pre-Conference Content (4 weeks prior)

Week -4: LinkedIn post by Hugo: "BIO 2026: what I want to talk about with pharma and biotech leaders." Sets agenda -- rare disease strategy, enrollment infrastructure, the discoverability gap. Not product-forward.

Week -3: BIO Connect meeting scheduling opened: Hugo and Elisa schedule 30-minute 1:1 meetings with 20 priority pharma and biotech executive contacts via the BIO meeting platform.

Week -2: LinkedIn post by Hugo: flagship article ("The Patient Is Already Searching") referenced as pre-read for BIO conversations. Establishes intellectual credibility.

Week -1: Personalised pre-conference emails to all scheduled BIO meetings: "Looking forward to our conversation at BIO. I've been thinking about [specific point relevant to their therapeutic focus area]."

During-Conference Content

Day 1: LinkedIn post by Hugo: "Day 1 at BIO. The rare disease enrollment conversation I'm looking forward to having." Photo.

Day 2: LinkedIn post by Elisa: one honest observation from a BIO session or conversation -- not branded, just insightful. Authentic BIO content from a clinical ops angle.

Partner meetings (1:1 scheduled): These conversations are the primary BIO deliverable. Hugo and Elisa each have 8-10 30-minute meetings. Materials available: company overview (1 page), TDI executive summary (2 pages), REFER tier comparison.

Post-Conference Follow-Up

Day after BIO: Personalised follow-up email from Hugo or Elisa (whichever met the contact) to all BIO meeting contacts. Specific, referencing what was discussed. Includes next-step suggestion (diagnostic call, TDI download, specific case study).

Week +1: LinkedIn post by Hugo: "What BIO told me about where rare disease development strategy is heading." Analysis post, not recap.

Week +2: BIO leads -- pharma/biotech executives -- entered into Sequence A (pharma sponsor awareness). CRO BD leads entered into Sequence B with BIO-specific personalisation.